

Behavioral Health Levels of Care Comparison Chart, Child or Adolescent

MCG Health
Behavioral Health Care
28th Edition

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Behavioral Health Levels of Care Comparison Chart, Child or Adolescent

Inpatient Treatment and Alternatives

Inpatient Behavioral Health Level of Care, Child or Adolescent BHG

- Admission to **Inpatient Level of Care for Child or Adolescent** (CALOCUS-CASII Level 6 - Medically Managed Residence Based Services, Composite Score 28 or more) is indicated due to **ALL** of the following[A][B][C][D]:
 - Patient risk or severity of behavioral health disorder is appropriate to proposed level of care, as indicated by **1 or more** of the following(1)(2)(3)(4)(6)(11)(12)(13)(14)(15):
 - ☐ Imminent danger to self for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(16)(17)(18):
 - Imminent risk for recurrence of Suicide attempt or act of serious Harm to self is present, as indicated by **ALL** of the following:
 - There has been a recent Suicide attempt or deliberate act of serious Harm to self.
 - There has not been Sufficient relief of the factors that precipitated attempt or act.
 - Current plan for suicide or serious Harm to self is present.
 - Command auditory hallucinations or paranoid delusions contributing to risk for suicide or serious Harm to self are present.
 - Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[E]:
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Severe conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as extremely Impaired impulse control, judgment, or insight; severe Mania (child or adolescent), severe unreliability, or extreme agitation with desperation
 - Ruminative flooding; uncontrollable and overwhelming negative thoughts
 - Frantic hopelessness; fatalistic conviction that life will not improve, along with oppressive

Residential Behavioral Health Level of Care, Child or Adolescent BHG

- Admission to **Residential Level of Care for Child or Adolescent** (CALOCUS-CASII Level 5 - Medically Monitored Residence Based Services, Composite Score 23 to 27) is indicated due to **ALL** of the following[B][C][T][U][V]:
 - Patient risk or severity of behavioral health disorder is appropriate to proposed level of care, as indicated by **1 or more** of the following(1)(2)(3)(4)(6)(11)(12)(13)(14)(15):
 - ☐ Danger to others for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(17)(18)(20):
 - Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another
 - Patient has persistent thoughts of homicide or serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[E]:
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight; moderate to moderately severe Mania (child or adolescent) or unreliability
 - Indication or report of significant physical or sexual aggression with Impaired impulse control, judgment, or insight that significantly endangers another
 - Current homicidal ideation with either clearly expressed intentions or past history of carrying out such behavior
 - ☐ Danger to self for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(16)(17)(18):
 - Command auditory hallucinations contributing to risk for suicide or serious Harm to self(16)
 - Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level

Observation Behavioral Health Level of Care BHG

- Admission to **Observation Level of Care** is indicated due to **ALL** of the following[C](7)(48)(49)(50)(51):
 - Care needed requires further evaluation, treatment, or disposition arrangement for behavioral health issues, as indicated by **1 or more** of the following(33)(52)(53)(54):
 - Imminent danger to self for adult or Imminent danger to self for child or adolescent
 - Imminent danger to others for adult or Imminent danger to others for child or adolescent
 - Severe** dysfunction in daily living for adult or **Severe** dysfunction in daily living for child or adolescent
 - Patient is experiencing withdrawal or other drug-induced disorder.[BB][CC](6)(60)(61)(62)(63)(64)
 - CIWA-Ar Calculator
 - COWS Calculator
 - ☐ Observation care is appropriate for anticipated scope and care needed for patient, as indicated by **ALL** of the following:
 - Care is beyond scope of usual outpatient care episode or symptoms are not adequately controlled despite emergency department care.[DD]
 - Care needed is appropriate for observation care, as indicated by **1 or more** of the following(48)(52)(65):
 - Diagnostic evaluation is needed and is not feasible in outpatient setting (eg, awaiting psychiatric evaluation which is not available in ambulatory care, or outpatient evaluation is inappropriate to

Crisis Intervention Behavioral Health Level of Care BHG

- Admission to **Crisis Intervention Level of Care** (LOCUS/CALOCUS-CASII Basic Services) is indicated due to **ALL** of the following[C][FF](17)(68)(71)(72)(73)(74)(75):
 - Around-the-clock behavioral care or close monitoring and intervention is indicated by **1 or more** of the following(45)(76)(77):
 - Danger to self for adult or Danger to self for child or adolescent
 - Danger to others for adult or Danger to others for child or adolescent
 - Serious** dysfunction in daily living for adult or **Serious** dysfunction in daily living for child or adolescent
 - ☐ Patient management/treatment at lower level of care is not feasible (eg, lower level of care is unavailable or is not appropriate for patient condition) until acute intervention or modification (eg, acute medication, identification of needed resources and support, change in living situation) is initiated.[GG]
 - ☐ Situation and expectations are appropriate for crisis intervention services, as indicated by **ALL** of the following(5)(6)(19)(32):
 - Recommended treatment is necessary, appropriate, and not feasible at lower level of care (eg, lower level of care is inappropriate for patient condition or is not available).[HH]
 - Sufficient treatment outcome (eg, stabilization and identification of resources and

sense of entrapment and doom ☐ Imminent danger to others for child or adolescent is present due to 1 or more of the following(1)(2)(3)(17)(19)(20): - Imminent risk for recurrence of attempt to seriously Harm another is present, as indicated by ALL of the following: - There has been a recent attempt to seriously Harm another. - There has not been Sufficient relief of factors that precipitated the attempt. - Current plan for homicide or serious Harm to another is present. - Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another are present. - Patient has persistent thoughts of homicide or violent acts that likely could result in serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by 1 or more of the following[E]: - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient - Severe conflict in family environment or other inadequacy in patient support system - Patient characteristics, such as extremely Impaired impulse control, judgment, or insight; severe Mania (child or adolescent), severe unreliability, or extreme agitation with desperation - Ruminative flooding; uncontrollable and overwhelming negative thoughts - Indication or report of repeated behavior, including physical or sexual aggression that is clearly injurious to others with history, plan or intent, and extremely Impaired impulse control, judgment, or insight ■ Behavioral health disorder is present and appropriate for inpatient care with ALL of the following: ☐ Severe psychiatric, behavioral, or other comorbid conditions are appropriate for management at proposed level of care, as indicated by 1 or more of the following[F][G]: ☐ Psychiatric symptoms, including 1 or more of the following(15): - Hallucinations (child or adolescent), Delusions (child or adolescent), or other Positive symptoms (child or adolescent) - Negative symptoms (child or adolescent), including social or emotional withdrawal or lack of self-initiated behavior or movement - Mania (child or adolescent) - Depressive symptoms (child or adolescent) - Anxiety (child or adolescent) - Hyperactivity or inattention (child or	of care as indicated by 1 or more of the following[E]: - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient - Conflict in family environment or other inadequacy in patient support system - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight; moderate to moderately severe Mania (child or adolescent) or unreliability - Indication or report of significant physical or sexual risky behavior with Impaired impulse control, judgment, or insight that significantly endangers self ■ Behavioral health disorder is present and appropriate for residential care with ALL of the following: ☐ Moderately severe psychiatric, behavioral, or other comorbid conditions are appropriate for management at proposed level of care, as indicated by 1 or more of the following[F][G]: ☐ Psychiatric symptoms, including 1 or more of the following(15): - Hallucinations (child or adolescent), Delusions (child or adolescent), or other Positive symptoms (child or adolescent) - Negative symptoms (child or adolescent), including social or emotional withdrawal or lack of self-initiated behavior or movement - Mania (child or adolescent) - Depressive symptoms (child or adolescent) - Anxiety (child or adolescent) - Hyperactivity or inattention (child or adolescent) - Obsessions or compulsions (child or adolescent) - Personality disorders/maladaptive personality traits - Other psychiatric symptoms related to underlying psychiatric disorder or underlying comorbid condition having a negative impact on primary psychiatric disorder ○ Comorbid biomedical or developmental condition[H][I] ○ Comorbid substance use disorder[J][K] ○ Cognitive or memory impairment[L] ○ Impaired impulse control, judgment, or insight ☐ Emotional or behavioral disturbances impacting symptoms or function, including 1 or more of the following[M](4)(6)(41): - Externalizing symptoms (eg, angry outbursts, physical or verbal aggression, oppositional defiant	patient condition). - Acute treatment and response evaluation is needed (eg, acute medication and observation for exacerbation of chronic psychosis). - Monitoring is needed while awaiting suitability for evaluation (eg, patient cannot be adequately assessed because of acute intoxication or substance-induced effect) or psychosocial management and arrangement for follow-up care. ■ Biopsychosocial stressors[P] potentially contributing to clinical presentation (eg, comorbidities,[K][Q] illness history, environment,[R] social network, ability to cope, and level of engagement[EE]) have been assessed and are absent or manageable at proposed level of care. (1)(2)(3)(4)(5)(6)(7)(8)(10)(46)	support for care outside of crisis intervention services) is expected within short time period (eg, 1 to 3 days). ■ Patient is willing to participate in treatment (or agrees to participate at direction of parent or guardian) within specified intervention and treatment structure voluntarily (or due to court order).[II] ■ There is no anticipated need for physical restraint, seclusion, or other involuntary control (eg, patient not actively violent). ■ There is no need for around-the-clock medical or nursing care. ■ Patient has sufficient ability to respond as planned to individual and group therapeutic interventions.[KK] ■ Biopsychosocial stressors[P] potentially contributing to clinical presentation (eg, comorbidities,[K][Q] illness history, environment,[R] social network, ability to cope, and level of engagement[EE]) have been assessed and are absent or manageable at proposed level of care. (1)(2)(3)(4)(5)(6)(7)(8)(10)(46)
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- adolescent)
- Obsessions or compulsions (child or adolescent)
- Personality disorders/maladaptive personality traits
- Other psychiatric symptoms related to underlying psychiatric disorder or underlying comorbid condition having a negative impact on primary psychiatric disorder
- Comorbid biomedical or developmental condition^{[H][I]}
- Comorbid substance use disorder^{[J][K]}
- Cognitive or memory impairment^[L]
- Impaired impulse control, judgment, or insight

☐ **Emotional or behavioral disturbances impacting symptoms or function, including 1 or more of the following^{[M](4)(6)(41):}**

- Externalizing symptoms (eg, angry outbursts, physical or verbal aggression, oppositional defiant traits, agitation, anxiety, or other disruptive behaviors)
- Internalizing symptoms (eg, sulking, rumination, anhedonia, dysphoria, apathy)
- Evidence of severely diminished ability to assess consequences of own actions (eg, acts of severe property damage)
- High levels of family conflict or interpersonal conflict
- Escalating relapse behaviors
- Other emotional or behavioral disturbance relevant to clinical presentation

☐ **Severe** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following^{(1)(2)(3)(8)(42)(43)(44):}

- Incapacitation because of grave disability (eg, severe regression with inability to provide for self)
- Extreme deterioration in interactions with peers, adults, or family (eg, assaultive behaviors with no provocation, high levels of family conflict)
- Complete withdrawal from all social interactions
- Complete neglect of self-care with associated impairment in physical status
- Extreme disruption in vegetative function (eg, life-sustaining functions such as eating)
- Nearly complete inability to maintain any appropriate school behavior or academic achievement
- Severely diminished ability to assess consequences of own actions (eg, acts of severe property damage)
- Other evidence of severe dysfunction

☐ Treatment services available at proposed level of care are necessary to meet patient needs and **1 or more** of the following^{[N]:}

- Specific condition related to admission diagnosis is present and judged likely

traits, agitation, anxiety, or other disruptive behaviors)

- Internalizing symptoms (eg, sulking, rumination, anhedonia, dysphoria, apathy)
- Evidence of severely diminished ability to assess consequences of own actions (eg, acts of severe property damage)
- High levels of family conflict or interpersonal conflict
- Escalating relapse behaviors
- Other emotional or behavioral disturbance relevant to clinical presentation

☐ **Serious** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following^{(1)(2)(3)(8)(42)(43)(44):}

- Serious deterioration in interpersonal interactions (eg, impulsive or abusive behaviors)
- Significant withdrawal and avoidance of almost all social interaction
- Consistent failure to achieve self-care as appropriate to age or developmental level
- Serious disturbance in vegetative status (eg, weight change, sleep disruption) threatening physical function
- Inability to perform adequately in school (including specialized setting) due to disruptive or aggressive behavior
- Severely diminished ability to assess consequences of own actions (eg, acts of severe property damage)
- Other evidence of serious dysfunction

☐ Treatment services available at proposed level of care are necessary to meet patient needs and **1 or more** of the following^{[N]:}

- Specific condition related to admission diagnosis is present and judged likely to further improve at proposed level of care.
- Specific condition related to admission diagnosis is present and judged likely to deteriorate in absence of treatment at proposed level of care.
- Patient is receiving continuing care (eg, transition of care from more or less intensive level of care).

☐ Situation and expectations are appropriate for residential care for child or adolescent, as indicated by **ALL** of the following^{[W](1)(2)(3)(4)(8)(13)(15):}

- Recommended treatment is necessary, appropriate, and not feasible at lower level of care (eg, less intensive level is unavailable or not suitable for patient condition or history).^[X]
- Very short-term crisis intervention and resource planning for further care at nonresidential level is unavailable or inappropriate.
- Patient is willing to participate in treatment within highly structured setting voluntarily (or agrees to participate at direction of parent or guardian, or attend due to court order).^{[Y][Z]}
- There is no anticipated need for physical restraint, seclusion, or other involuntary treatment interventions (eg, patient not actively violent).
- Medical or nursing care services to address primary admission diagnosis are available, as indicated by **1 or more** of the following:

to further improve at proposed level of care.

- Specific condition related to admission diagnosis is present and judged likely to deteriorate in absence of treatment at proposed level of care.
- Patient is receiving continuing care (eg, transition of care from less intensive level of care).

☐ Situation and expectations are appropriate for inpatient care for child or adolescent, as indicated by **1 or more** of the following(1)(2)(3)(4)(8)(15)(19)(45):

- Patient is unwilling to participate voluntarily in treatment and requires treatment (eg, legal commitment or order by guardian) in an involuntary unit.
- Voluntary treatment at lower level of care is not feasible (eg, very short-term crisis intervention or residential care unavailable or insufficient for patient condition).
- Need for physical restraint, seclusion, or other involuntary treatment intervention is present (eg, actively violent patient for whom treatment in an involuntary unit is deemed necessary in accordance with applicable medical and legal criteria).
- Around-the-clock medical and nursing care to address symptoms and initiate intervention is required; specific need is identified.[O]
- Patient management/treatment at lower level of care is not feasible or is inappropriate (eg, less intensive level of care is unavailable or not suitable for patient condition or treatment history).
- Biopsychosocial stressors[P] potentially contributing to clinical presentation (eg, comorbidities,[K][Q] illness history, environment,[R] social network, ability to cope, and level of engagement[S]) have been assessed and are absent or manageable at proposed level of care. (1)(2)(3)(4)(5)(6)(7)(8)(10)(46)

- No anticipated need for around-the-clock medical or nursing monitoring (ie, comorbid medical, psychiatric, or behavioral conditions are absent or are of minimal severity, and are not expected to interfere with recovery)
- Active (but not around-the-clock) monitoring of patient by staff needed, and medical or nursing care can easily be provided if need arises (ie, comorbid medical, psychiatric, or behavioral conditions have potential to distract from treatment)
- Around-the-clock medical or nursing monitoring needed, but intensive treatment and resources of licensed hospital are not anticipated (ie, due to severity of primary admitting diagnosis, or presence of active comorbid medical, psychiatric, or behavioral conditions that are distracting from treatment)
- Patient has sufficient ability to respond as planned to individual and group therapeutic interventions.[AA]
- Biopsychosocial stressors[P] potentially contributing to clinical presentation (eg, comorbidities,[K][Q] illness history, environment,[R] social network, ability to cope, and level of engagement[S]) have been assessed and are absent or manageable at proposed level of care. (1)(2)(3)(4)(5)(6)(7)(8)(10)(46)

Outpatient Treatment Alternatives

Partial Hospital Behavioral Health Level of Care, Child or Adolescent [BHG](#)

- Admission to **Partial Hospital Program Level of Care for Child or Adolescent** (CALOCUS-CASII Level 4 - Medically Monitored Community Based Services, Composite Score 20 to 22) is indicated due to **ALL** of the following[B][C][LL][MM][NN][OO]:
 - Patient risk or severity of behavioral health disorder is appropriate to proposed level of care, as indicated by **1 or more** of the following[P][P](1)(2)(3)(4)(6)(10)(11)(12)(13)(14)(15):
☐ Danger to self for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(16)(17)(18):
 - Command auditory hallucinations contributing to risk for suicide or serious Harm to self(16)
 - Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level of care as indicated by **1 or more** of the following[E]:
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or

Intensive Outpatient Program Behavioral Health Level of Care, Child or Adolescent [BHG](#)

- Admission to **Intensive Outpatient Program Level of Care for Child or Adolescent** (CALOCUS-CASII Level 3 - High Intensity Community Based Services, Composite Score 17 to 19) is indicated due to **ALL** of the following[B][C][MM][WW][NN][OO]:
 - Patient risk or severity of behavioral health disorder is appropriate to proposed level of care, as indicated by **1 or more** of the following[P][P](1)(2)(3)(4)(6)(10)(11)(12)(13)(14)(15):
☐ Danger to self for child or adolescent is present but low, as indicated by **ALL** of the following(1)(2)(3)(16):
 - Absent or minimal Thoughts of suicide or serious Harm to self
 - Absent or low intent to act on thoughts of Harm
 - No current plan is present.
 - ☐ Danger to others for child or adolescent is present but low, as indicated by **ALL** of the following(1)(2)(3)(20):
 - Absent or minimal thoughts of homicide or serious Harm to another
 - Absent or low intent to act on thoughts of Harm
 - No current plan is present.
 - Behavioral health disorder is present and appropriate for intensive outpatient program care with **ALL** of the following:
☐ **Mild to moderate** psychiatric, behavioral, or other comorbid conditions are appropriate for management at proposed level of care, as indicated by **1 or more** of

Outpatient Behavioral Health Level of Care, Child or Adolescent [BHG](#)

- Admission to **Outpatient Level of Care for Child or Adolescent** (CALOCUS-CASII Level 2 - Low Intensity Community Based Services, Composite Score 14 to 16) is indicated due to **ALL** of the following[Z][Z][AAA][C][OO](1)(2)(3):
 - Patient risk or severity of behavioral health disorder is appropriate to proposed level of care and patient has active symptoms that require ongoing treatment, as indicated by **ALL** of the following(6)(7)(10)(15)(80)(81):
☐ **Mild** psychiatric, behavioral, or other comorbid conditions are appropriate for management at proposed level of care, as indicated by **1 or more** of the following[G]:
☐ Psychiatric symptoms, including **1 or more** of the following(15):
 - Hallucinations (child or adolescent), Delusions (child or adolescent), or other Positive symptoms (child or adolescent)
 - Negative symptoms (child or adolescent), including social or emotional withdrawal or lack of self-initiated behavior or movement
 - Mania (child or adolescent)
 - Depressive symptoms (child or adolescent)

Day Treatment Behavioral Health Level of Care [BHG](#)

- Admission to **Day Treatment Level of Care** is indicated due to **ALL** of the following[C][CCC][DDD]:
 - Behavioral health disorder is present and patient symptom severity and functional status is appropriate to proposed level of care, as indicated by **ALL** of the following[EEE][FFF](5)(6)(7)(10)(32)(70):
 - **Mild to moderate** Psychiatric, behavioral, or other comorbid conditions for adult or **Mild to moderate** Psychiatric, behavioral, or other comorbid conditions for child or adolescent [GGG][HHH][III]
 - **Mild to moderate** dysfunction in daily living for adult or **Mild to moderate** dysfunction in daily living for

insight; moderate to moderately severe Mania (child or adolescent) or unreliability - Indication or report of significant physical or sexual risky behavior with Impaired impulse control, judgment, or insight that significantly endangers self ☐ Danger to others for child or adolescent is present due to 1 or more of the following(1)(2)(3)(17)(18)(20): - Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another - Patient has persistent thoughts of homicide or serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by 1 or more of the following[E]: - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient - Conflict in family environment or other inadequacy in patient support system - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight that significantly endangers another - Current homicidal ideation with either clearly expressed intentions or past history of carrying out such behavior ■ Behavioral health disorder is present and appropriate for partial hospital program care with ALL of the following: ☐ Moderate psychiatric, behavioral, or other comorbid conditions are appropriate for management at proposed level of care, as indicated by 1 or more of the following[F][G]: ☐ Psychiatric symptoms, including 1 or more of the following(15): - Hallucinations (child or adolescent), Delusions (child or adolescent), or other Positive symptoms (child or adolescent) - Negative symptoms (child or adolescent), including social or emotional withdrawal or lack of self-initiated behavior or movement - Mania (child or adolescent) - Depressive symptoms (child or adolescent) - Anxiety (child or adolescent)	the following[F][G]: ☐ Psychiatric symptoms, including 1 or more of the following(15): - Hallucinations (child or adolescent), Delusions (child or adolescent), or other Positive symptoms (child or adolescent) - Negative symptoms (child or adolescent), including social or emotional withdrawal or lack of self-initiated behavior or movement - Mania (child or adolescent) - Depressive symptoms (child or adolescent) - Anxiety (child or adolescent) - Hyperactivity or inattention (child or adolescent) - Obsessions or compulsions (child or adolescent) - Personality disorders/maladaptive personality traits - Other psychiatric symptoms related to underlying psychiatric disorder or underlying comorbid condition having a negative impact on primary psychiatric disorder - Comorbid biomedical or developmental condition[H][I] - Comorbid substance use disorder[J][K] - Cognitive or memory impairment[L] - Impaired impulse control, judgment, or insight ☐ Emotional or behavioral disturbances impacting symptoms or function, including 1 or more of the following[M](4)(6)(41): - Externalizing symptoms (eg, angry outbursts, physical or verbal aggression, oppositional defiant traits, agitation, anxiety, or other disruptive behaviors) - Internalizing symptoms (eg, sulking, rumination, anhedonia, dysphoria, apathy) - Evidence of severely diminished ability to assess consequences of own actions (eg, acts of severe property damage) - High levels of family conflict or interpersonal conflict - Escalating relapse behaviors - Other emotional or behavioral disturbance relevant to clinical presentation ☐ Mild to moderate dysfunction in daily living for child or adolescent is present, as indicated by 1 or more of the following(1)(2)(3)(8)(42)(43)(44): - Some difficulty in relationships with peers, family, or adults (eg,	- Anxiety (child or adolescent) - Hyperactivity or inattention (child or adolescent) - Obsessions or compulsions (child or adolescent) - Personality disorders/maladaptive personality traits - Other psychiatric symptoms related to underlying psychiatric disorder or underlying comorbid condition having a negative impact on primary psychiatric disorder - Comorbid biomedical or developmental condition[H][I] - Comorbid substance use disorder[J][K] - Cognitive or memory impairment[L] - Impaired impulse control, judgment, or insight ☐ Emotional or behavioral disturbances impacting symptoms or function, including 1 or more of the following[M](4)(6)(41): - Externalizing symptoms (eg, angry outbursts, physical or verbal aggression, oppositional defiant traits, agitation, anxiety, or other disruptive behaviors) - Internalizing symptoms (eg, sulking, rumination, anhedonia, dysphoria, apathy) - Evidence of severely diminished ability to assess consequences of own actions (eg, acts of severe property damage) - High levels of family conflict or interpersonal conflict - Escalating relapse behaviors - Other emotional or behavioral disturbance relevant to clinical presentation ☐ Mild dysfunction in daily living for child or adolescent is present, as indicated by 1 or more of the following(1)(2)(3)(8)(42)(43)(44): - Minor deterioration or episodic failure in relationships with peers, family, or adults (eg, defiance, lying, provocative behavior) - Self-care sporadically below usual or expected standards - Impairments in function characterized by ability to show improvements following episodes of deterioration - Diminished ability to assess consequences of own actions (eg, acts of severe property damage) that therapeutic intervention can remediate - Self-neglect with at least some minor limitation in ability to provide for basic needs (by self or others) in maintenance care	child or adolescent ☐ Patient condition does not require urgent intervention, as indicated by ALL of the following[J][J]: - Signs or symptoms related to admitting diagnosis (or impact of comorbidity on admitting diagnosis) are stable or improving and there has not been an acute increase in condition severity. - Impairments in function are stable or improving and there has not been a recent substantial deterioration in function. ☐ Treatment services available at proposed level of care are necessary to meet patient needs and 1 or more of the following[K][K]: - Specific condition related to admission diagnosis is present and judged likely to further improve at proposed level of care. - Specific condition related to admission diagnosis is present and judged likely to deteriorate in absence of treatment at proposed level of care. - Patient is receiving continuing care (eg, transition of care from more or less intensive level of care). ☐ Situation and expectations are appropriate for day treatment services, as indicated by ALL of the following(4)(5)(6)(32): - Recommended treatment is necessary and appropriate[L][L] and not feasible with less intensive intervention (eg, less intensive support, such as outpatient care, is not suitable to patient condition or history).[H][H] - Patient and family or caregivers, as appropriate, are willing to participate in treatment voluntarily.[M][J][J]
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■ Hyperactivity or inattention (child or adolescent) ■ Obsessions or compulsions (child or adolescent) ■ Personality disorders/maladaptive personality traits ■ Other psychiatric symptoms related to underlying psychiatric disorder or underlying comorbid condition having a negative impact on primary psychiatric disorder	○ Comorbid biomedical or developmental condition^[H] [I] ○ Comorbid substance use disorder^[J] [K] ○ Cognitive or memory impairment^[L] ○ Impaired impulse control, judgment, or insight	○ Defiance, lying), but without physical aggression ○ Self-care sometimes below usual or expected standards ○ Disturbance in vegetative status (eg, eating, sleeping habits) that does not pose serious threat to health ○ Deteriorating school behavior such that disruption in school is threatened (eg, threat of expulsion) ○ Diminished ability to assess consequences of own actions (eg, acts of severe property damage) that therapeutic intervention can remediate ○ Other evidence of mild to moderate dysfunction	• Other evidence of mild dysfunction	■ Patient has sufficient ability to respond as planned to individual and group therapeutic interventions. ^[KK](91)(92) ■ Biopsychosocial stressors^[P] potentially contributing to clinical presentation (eg, comorbidities,^[K] ^[Q] illness history, environment,^[R] social network, ability to cope, and level of engagement^[EE]) have been assessed and are absent or manageable at proposed level of care. (1)(2)(3)(4)(5)(6)(7)(8)(10)(46)
☐ Emotional or behavioral disturbances impacting symptoms or function, including 1 or more of the following^[M](4)(6)(41): ■ Externalizing symptoms (eg, angry outbursts, physical or verbal aggression, oppositional defiant traits, agitation, anxiety, or other disruptive behaviors) ■ Internalizing symptoms (eg, sulking, rumination, anhedonia, dysphoria, apathy) ■ Evidence of severely diminished ability to assess consequences of own actions (eg, acts of severe property damage) ■ High levels of family conflict or interpersonal conflict ■ Escalating relapse behaviors ■ Other emotional or behavioral disturbance relevant to clinical presentation	☐ Moderate dysfunction in daily living for child or adolescent is present, as indicated by 1 or more of the following⁽¹⁾⁽²⁾⁽³⁾⁽⁸⁾⁽⁴²⁾⁽⁴³⁾⁽⁴⁴⁾: ○ Troubled relationships with peers, family, or adults (eg, conflicted, withdrawn), but without physical aggression ○ Self-care frequently below usual or expected standards ○ Significant disturbance in vegetative status (eg, eating, sleeping habits), but not posing serious threat to health ○ Deteriorated school behavior such that disruption in school has occurred (eg, suspension, expulsion), and child is at risk for placement in alternative school setting or repeating grade ○ Diminished ability to assess consequences of own actions (eg, acts of severe property damage) ○ Other evidence of moderate dysfunction	☐ Treatment services available at proposed level of care are necessary to meet patient needs and 1 or more of the following^[N]: ■ Specific condition related to admission diagnosis is present and judged likely to further improve at proposed level of care. ■ Specific condition related to admission diagnosis is present and judged likely to deteriorate in absence of treatment at proposed level of care. ■ Patient is receiving continuing care (eg, transition of care from more or less intensive level of care). ☐ Situation and expectations are appropriate for intensive outpatient program for child or adolescent, as indicated by ALL of the following^{[MM][RR][XX]}(1)(2)(3)(4)(8)(13)(15): ■ Recommended treatment is necessary, appropriate, and not feasible at lower level of care (eg, less intensive level is unavailable or not suitable to patient condition or history).^[SS] ■ Patient is willing to participate in treatment voluntarily, or at least agrees to participate at direction of parent or guardian.^{[TT][Y][Z]} ■ Patient has sufficient ability to respond as planned to individual and group therapeutic interventions.^[AA] ^[VV] ■ Biopsychosocial stressors^[YY] potentially contributing to clinical presentation (eg, comorbidities,^[K] ^[Q] illness history, environment,^[R] social network, ability to cope, and level of engagement^[S]) have been assessed and are absent or manageable at proposed level of care. (1)(2)(3)(4)(5)(6)(7)(8)(10)(46)	☐ Treatment services available at proposed level of care are necessary to meet patient needs and 1 or more of the following^[N]: ■ Specific condition related to admission diagnosis is present and judged likely to further improve at proposed level of care. ■ Specific condition related to admission diagnosis is present and judged likely to deteriorate in absence of treatment at proposed level of care. ■ Patient is receiving continuing care (eg, transition of care from more or less intensive level of care). ☐ Situation and expectations are appropriate for outpatient care for child or adolescent, as indicated by ALL of the following^[BB](1)(2)(3)(4)(13)(15): ■ Recommended treatment is necessary and appropriate, given patient condition or history.^[X] ■ Patient is willing to participate in treatment voluntarily (or agrees to participate at direction of parent or guardian).^{[Y][Z]} ■ Patient has sufficient ability to respond as planned to individual and group therapeutic interventions.^[AA] ■ Biopsychosocial stressors^[P] potentially contributing to clinical presentation (eg, comorbidities, ^[K] ^[Q] illness history, environment,^[R] social network, ability to cope, and level of engagement^[S]) have been assessed and are absent or manageable at proposed level of care. (1)(2)(3)(4)(5)(6)(7)(8)(10)(46)	☐ Treatment services available at proposed level of care are necessary to meet patient needs and 1 or more of the following^[N]: ■ Specific condition related to admission diagnosis is present and judged likely to further improve at proposed level of care. ■ Specific condition related to admission diagnosis is present and judged likely to deteriorate in absence of treatment at proposed level of care. ■ Patient is receiving continuing care (eg, transition of care from more or less intensive level of care). ☐ Situation and expectations are appropriate for outpatient care for child or adolescent, as indicated by ALL of the following^[BB](1)(2)(3)(4)(13)(15): ■ Recommended treatment is necessary and appropriate, given patient condition or history.^[X] ■ Patient is willing to participate in treatment voluntarily (or agrees to participate at direction of parent or guardian).^{[Y][Z]} ■ Patient has sufficient ability to respond as planned to individual and group therapeutic interventions.^[AA] ■ Biopsychosocial stressors^[P] potentially contributing to clinical presentation (eg, comorbidities, ^[K] ^[Q] illness history, environment,^[R] social network, ability to cope, and level of engagement^[S]) have been assessed and are absent or manageable at proposed level of care. (1)(2)(3)(4)(5)(6)(7)(8)(10)(46)

☐ Treatment services available at proposed level of care are necessary to meet patient needs and **1 or more** of the following[N]:

- Specific condition related to admission diagnosis is present and judged likely to further improve at proposed level of care.
- Specific condition related to admission diagnosis is present and judged likely to deteriorate in absence of treatment at proposed level of care.
- Patient is receiving continuing care (eg, transition of care from more or less intensive level of care).

☐ Situation and expectations are appropriate for partial hospital program for child or adolescent, as indicated by **ALL** of the following[MM][QQ][RR](1)(2)(3)(4)(8)(10)(13)(15):

- Recommended treatment is necessary, appropriate, and not feasible at lower level of care (eg, less intensive level is unavailable or not suitable to patient condition or history).[SS]
- Patient is willing to participate in treatment voluntarily (or at direction of parent or guardian).[Y][TT][Z]
- There is no anticipated need for physical restraint or other involuntary treatment interventions.
- There is no need for around-the-clock medical or nursing care.
- There has been no recent attempt or Current plan to seriously Harm another.
- There has been no recent Suicide attempt or act of serious Harm to self, or there has been Sufficient relief of precipitants of any such action.
- There is no Current plan for suicide or serious Harm.
- Patient's family, caregiver, guardian, or other available resources can adequately monitor patient condition when away from partial hospital program (ie, partial hospital program resources are sufficient to address any inadequacies in support system).[UU]
- Patient, family, caregiver, or guardian will contact care provider if Current plan for suicide or serious Harm develops.[TT](10)(18)
- Patient has sufficient ability to respond as planned to individual and group therapeutic interventions.[AA][VV]
- Biopsychosocial stressors[P] potentially contributing to clinical presentation (eg, comorbidities,[K][Q] illness history, environment,[R] social network, ability to cope, and level of engagement[S]) have been assessed and are absent or manageable at proposed level of care.(1)(2)(3)(4)(5)(6)(7)(8)(10)(46)

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Footnotes

[A] Inpatient psychiatric units generally are locked, equipped to restrain or seclude patients for safety if necessary, and staffed by nurses around the clock. A covering physician is available to assess patients if indicated around the clock.(1)(2)(3)(4) [A in Context Link 1]

[B] Symptoms or conditions used to determine the appropriate treatment intensity should be due to the underlying behavioral health diagnosis or represent factors that contribute to destabilization of the underlying diagnosis, and are acute in nature or represent a significant worsening over baseline.(1)(2)(3) [B in Context Link 1, 2, 3, 4]

[C] The purpose of the care guidelines is to promote evidence-based care across the continuum of care to enhance the delivery of quality healthcare. Indications are presented for different levels of care. These indications help define the optimal level of care and can assist in developing alternatives to higher levels of care, tracking patient progress during treatment within a level of care, facilitating the progress of patients whose recovery is delayed, and preparing comprehensive plans for transition of patients from one level of care to another. Relevant professional society guidelines are foundational content for evaluation and treatment of behavioral health disorders at different levels of care and are complemented by the best available published evidence.(1)(2)(3)(5)(6)(7)(8)(9)(10) [C in Context Link 1, 2, 3, 4, 5, 6, 7, 8]

[D] Composite score does not replace clinical judgment and is meant as a guide to assist in determining needed level of care. Extreme risk of harm, severe functional impairment, or severe comorbidity may independently necessitate placement at inpatient level of care.(1)(2)(3) [D in Context Link 1]

[E] Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(16) [E in Context Link 1, 2, 3, 4, 5, 6]

[F] Determination of severity may be based on symptom severity alone (including intensity or frequency of symptoms and the extent to which they interfere with functioning), or may be due to other factors (eg, comorbid medical illness, developmental condition, cognitive impairment, substance use disorder, or other factors that contribute to destabilization or decreased ability to cope with the underlying behavioral health disorder). If the comorbidity affects the level of care appropriate to meet the patient's behavioral health needs, the manner in which it impacts the behavioral health condition (and how the comorbidity will be managed at the proposed level of care) should be documented in order to optimize comprehensive patient care. Assessment for comorbid conditions should take place at admission

evaluation, and any identified conditions should be manageable by the program (either directly or through alternative arrangements). Co-occurring mental health and substance use conditions and co-occurring medical and behavioral health conditions are considered an expectation, not an exception, in all programs at all psychiatric and addiction service levels of care. Integrated care, with recovery-oriented and "co-occurring capable" services, is increasingly expected to be included in the design of programs at all levels of care and in all settings. In programs without integrated care, comorbidities that require more active management or that currently distract the patient from treatment or have been associated with challenges in treatment follow-through in the past should prompt consideration of a program designed to provide treatment for both disorders (eg, a dual-diagnosis program).(1)(2)(3)(6)(15) [F in Context Link 1, 2, 3, 4]

[G] The diagnosis and severity of depressive disorders may be confirmed and documented by the use of standardized rating scales that reliably measure depressive symptoms in children and adolescents, including the Beck Depression Inventory (BDI), the Children's Depression Inventory (CDI), and the Patient Health Questionnaire-9 (PHQ-9) modified for adolescents (PHQ-A) in the primary care setting.(21)(22)(23) As an example, the PHQ-9 asks the patient to describe the frequency of 9 symptoms over the past 2 weeks on a scale of 0 to 3 (0 = not at all, 1 = several days, 2 = more than half the days, 3 = nearly every day), and depression severity is scored as follows: 0 to 4 none, 5 to 9 mild, 10 to 14 moderate, 15 to 19 moderately severe, and 20 to 27 severe.(24) In addition to screening, rating scales are often administered throughout treatment to measure changes in symptoms. An approximate 50% decrease in scores on the BDI and PHQ-A has been found to be clinically significant when reassessing an individual for interval improvement.(21)(25)(26) [G in Context Link 1, 2, 3, 4, 5]

[H] A significant comorbid biomedical or developmental condition has the potential to prolong the course of illness, or necessitate admission to a higher level of care or closer monitoring of patients requiring psychiatric care.(1)(2)(3)(5)(27) [H in Context Link 1, 2, 3, 4, 5]

[I] Assessment and treatment of co-occurring medical and/or developmental conditions through services and treatment settings capable of rendering integrated care is recommended.(1)(2)(3)(5)(7)(8) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(28)(29)(30)(31) The level that comorbid medical and/or developmental conditions are present may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care).(1)(2)(3)(5)(32) [I in Context Link 1, 2, 3, 4, 5]

[J] A significant comorbid substance use disorder (either reflecting comorbidity in a patient whose primary diagnosis is a psychiatric disorder, such as an affective disorder, or is present in a patient with a polysubstance use disorder) has the potential to impact the appropriate level of care in which the underlying condition is managed, or require consideration of a program with structures in place to treat multiple conditions, such as a dual-diagnosis program. The risk for problems with substance use is increased with the presence of many types of behavioral disturbance (eg, impulse control disorders, or self-medication of depression or anxiety symptoms). The severity of the substance use disorder as it relates to the underlying psychiatric condition includes the frequency and chronicity of use, ability to control use, intensity of other symptoms (eg, cravings, withdrawal), or other negative impact on the underlying condition.(1)(2)(3)(5)(6)(33) [J in Context Link 1, 2, 3, 4, 5]

[K] Evaluation/assessment and treatment of co-occurring substance use disorders through services and treatment settings capable of rendering integrated care is recommended.(1)(2)(3)(5)(7) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(28)(29)(30)(31) The level that comorbid substance use disorders contribute to the primary presenting condition may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care or specially dual-diagnosis program).(1)(2)(3)(5)(32) [K in Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13]

[L] Intake and assessment should include a mental status examination and memory assessment. If cognitive impairments are identified, they may require modifying the intensity or pace of planned interventions to allow the patient to participate in the program, or using an enhanced program designed to address co-occurring cognitive disorders.(33)(34)(35)(36)(37)(38) [L in Context Link 1, 2, 3, 4, 5]

[M] Symptoms may be circumscribed (eg, attention-seeking behaviors) or may be seen in the context of one or more major psychiatric disorders, including depression; anxiety disorders; obsessive-compulsive disorder; posttraumatic stress disorder; eating disorders; attention-deficit hyperactivity disorders; bipolar disorders; schizophrenia; personality disorders; autism spectrum disorder and other disruptive, impulse-control, and conduct disorders.(39)(40)(41) [M in Context Link 1, 2, 3, 4, 5]

[N] Assessment of the likelihood of benefit at the proposed level of care may help identify subgroups of patients more likely to respond to particular treatments and help optimize the care plan to increase the likelihood of successful recovery. It may also help determine if a specialized treatment setting (eg, dual-diagnosis program) or a different level of service intensity would be more appropriate to address the patient's needs.(1)(2)(3)(6) [N in Context Link 1, 2, 3, 4, 5]

[O] Examples of medical conditions appropriate for inpatient care include conditions that require intensive, around-the-clock medical monitoring and daily nursing interventions, or patients with significant metabolic or ECG abnormalities related to vomiting.(1)(2)(3) [O in Context Link 1]

[P] Biopsychosocial stressors may impact the level of care necessary to manage a psychiatric or behavioral condition, including the ability of the program to meet comprehensive patient needs, ensure treatment adherence, enhance motivation, or prevent relapse (ie, comorbidities, environmental factors, or other barriers may prevent effective treatment at a less intensive level of care than might otherwise be appropriate to the patient's condition). Biopsychosocial assessment factors should be incorporated into care planning, including planned treatment goals, and intensity and duration of interventions. A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(28)(29)(30)(31) Any identified deficits should be manageable by the program directly or through alternative arrangements.(1)(2)(3)(5) [P in Context Link 1, 2, 3, 4, 5, 6, 7]

[Q] Comorbid conditions may directly impact the experience of psychiatric symptoms (eg, COPD and anxiety), or may indirectly impact determination of the appropriate venue for care (eg, routine blood sugar and insulin management in people with diabetes). If clinically appropriate, testing related to diagnosis or management (eg, screening for liver and kidney function, hepatitis, HIV, syphilis, tuberculosis) may be performed off-site.(1)(2)(3)(5) Assessment and treatment of co-occurring medical and/or developmental conditions through services and treatment settings capable of rendering integrated care is recommended.(1)(2)(3)(5)(7)(8) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(28)(29)(30)(31) The level that comorbid medical and/or developmental conditions are present may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care).(1)(2)(3)(5)(32) [Q in Context Link 1, 2, 3, 4, 5, 6, 7, 8]

[R] The degree of environmental stressors and amount of support in the patient recovery environment should be considered in the context of the clinical presentation in determining the appropriate level of care for treatment. The level of environmental stressors may be low, mild, moderate, high, or extreme. The level of support in the environment may range from absent or minimal to limited to supportive or highly supportive.(1)(2)(3)(5)(7)(8)(32) [R in Context Link 1, 2, 3, 4, 5, 6, 7, 8]

[S] Participation motivated by a wish to avoid negative consequences rather than accept the need to work toward recovery may require more intense monitoring and follow-up.(6)(32)(33) The patient's level of engagement may be described on a continuum: optimal, positive, limited, minimal, or unengaged. Readiness to change may range from actively and willingly engaged to unable to follow treatment recommendations due to clinical condition.(1)(2)(3)(5)(32) [S in Context Link 1, 2, 3, 4, 5]

[T] Residential care is intended for patients who need around-the-clock behavioral care but do not need the level of physical security and high frequency of psychiatric and medical intervention that are available on an inpatient unit.(1)(2)(3)(4) [T in Context Link 1]

[U] Though state statutes sometimes describe "boot camps" or "wilderness therapy programs" as residential treatment centers, these programs often do not offer the types of services that would be characteristic of a clinical residential treatment center. The American Academy of Child and Adolescent Psychiatry indicates programs that are appropriately classified as clinical residential treatment centers include the use of multidisciplinary teams that include psychologists, psychiatrists, pediatricians, and licensed therapists who consistently are involved in the patient's care. Residential care staffing should include (but not be limited to) a child and adolescent psychiatrist (for adolescent programs, an adult psychiatrist with experience in treatment of this age group may be appropriate), as well as the ability to provide medical care (ill and preventive care) by a qualified primary care provider who is available 24 hours a day, with hospital resources identified as appropriate.(47) [U in Context Link 1]

[V] Composite score does not replace clinical judgment and is meant as a guide to assist in determining needed level of care. Serious risk of harm may independently necessitate placement in residential level of care. Serious functional impairment or major comorbidity may independently necessitate placement at residential level of care unless the patient has minimal stress in the recovery environment and high levels of support.(1)(2)(3) [V in Context Link 1]

[W] Examples of medical conditions appropriate for residential care include conditions that require active monitoring (either by patient or staff) and have the potential to distract from treatment of the admitting diagnosis, but are not so severe that they require the intensive level of monitoring provided in inpatient treatment.(5) [W in Context Link 1]

[X] Goals for care may be diagnostic, therapeutic, or rehabilitative in nature. Goals should be agreed upon by the patient (or guardian, as appropriate), and incorporate the patient's perspective on current problems, as well as the patient's specific values and preferences. Lack of progress toward goals should trigger a reassessment of the care plan (eg, determination as to accuracy of the admitting diagnosis, identification of barriers toward progress, incorporation of new problems that may have developed into the treatment plan, assessment of the appropriateness of initial management strategies), and lead to the modification of the plan as appropriate to promote optimal recovery.(1)(2)(3) [X in Context Link 1, 2]

[Y] Evaluation of the lack of willingness to participate in treatment (eg, refusal to consent to or participate in treatment) should ascertain whether or not additional efforts to engage the patient in treatment are likely to be successful. It should also include a risk assessment (eg, whether or not the patient would represent a risk to self or others), as well as determine whether or not pursuing involuntary treatment is indicated.(1)(2)(3) [Y in Context Link 1, 2, 3, 4]

[Z] Impairments in motivation to participate in treatment, limitations to engagement in care, and resistance to change are common in patients with mental health and substance use disorders, and may represent a feature of the disease process. A lack of motivation may indicate the need for more intensive services in order to help promote recovery or behavior change. Deficits in motivation or resistance to change should be addressed with therapies designed to enhance motivation to participate in treatment and work toward recovery (eg, motivational enhancement therapy). The level of motivation and goals/strategies designed to address motivation should be included in the care plan, and a lack of progress toward goals should trigger a reassessment of the care plan (eg, identification of barriers toward progress, incorporation of new problems that may have developed into the treatment plan, assessment of the appropriateness of initial management strategies), and lead to modification of the plan as appropriate to promote optimal recovery.(1)(2)(3) [Z in Context Link 1, 2, 3, 4]

[AA] A variety of factors may impact the ability of the patient to respond to treatment, including language, cognitive, and intellectual capacity, as well as the presence of any developmental conditions. Assessment of the ability to respond to planned interventions may help identify subgroups of patients more likely to respond to particular treatments, and help optimize (or modify) the care plan to increase the likelihood of successful recovery. It may also help determine if a specialized treatment setting (eg, dual-diagnosis program) or alternative level of care would be more appropriate to address the patient's needs. Patients who do not initially respond to scheduled therapeutic interventions may respond to modifications of the treatment plans or strategies designed to enhance ability. These include the addition or substitution of a new medication or psychosocial therapy, adjustment of the medication dose, increase in service intensity (some patients require higher levels of service intensity to provide reinforcement to enhance motivation and promote change), or decrease in service intensity (some patients may find higher levels of service intensity so restrictive as to be burdensome and hinder recovery).(1)(2)(3) [AA in Context Link 1, 2, 3, 4]

[BB] The Clinical Institute Withdrawal Assessment tool (CIWA) is used to assess 10 common withdrawal signs and symptoms, including: nausea/vomiting, tremor, anxiety, agitation, paroxysmal sweats; orientation and clouding of sensorium; tactile, auditory, and visual disturbances; and headache. The cumulative score is used to guide treatment management for patients undergoing withdrawal, and ranges from 0 to 67. Withdrawal severity may be categorized by score: mild (less than 10), moderate (10 to 18), severe (19 or greater with the absence of seizure, delirium, or hallucinations), or complicated (19 or greater with the presence of seizure, delirium, or new-onset hallucinations).(55)(56)(57) [BB in Context Link 1]

[CC] The Clinical Opiate Withdrawal Scale (COWS) is used by clinicians to measure 11 commonly observed opiate withdrawal signs or symptoms, including: resting pulse, sweating, restlessness, pupil size, bone or joint aches, runny nose or tearing, gastrointestinal upset, tremor, yawning, anxiety or irritability, and gooseflesh skin. Cumulative scores are described as mild (5 to 12), moderate (13 to 24), moderately severe (25 to 36), and severe (greater than 36). The COWS was developed to guide buprenorphine induction, and can be used to track opiate withdrawal symptoms over time or in response to treatment.(58)(59) [CC in Context Link 1]

[DD] Observation care can be provided in a psychiatric emergency services unit, holding unit, or other facility-based setting; or in a dedicated observation care unit or other hospital-based setting.(65)(66) [DD in Context Link 1]

[EE] Participation motivated by a wish to avoid negative consequences rather than accept the need to work toward recovery may require more intense monitoring and follow-up.(6)(32)(33) The patient's level of engagement may be described on a continuum: optimal, positive, limited, minimal, or unengaged. Readiness to change may range from actively and willingly engaged to unable to follow treatment recommendations due to clinical condition.(1)(2)(3)(5)(32) [EE in Context Link 1, 2, 3]

[FF] Short-term (1 to 3 days) crisis stabilization can be accomplished by crisis intervention care that can be done in a facility setting (eg, residential care, short-term psychiatric unit) or by outreach teams. (48)(67)(68) For voluntary patients with acute manifestations of psychiatric disorders who are being considered for inpatient admission because of clinical urgency but are not expected to require physical restraint or around-the-clock nursing care, randomized controlled trials have shown that a residential setting can provide treatment that is no less effective than inpatient care.(67)(69)(70) [FF in Context Link 1]

[GG] Acute interventions may address multiple factors contributing to a patient's condition including medication needs, identification of needed resources and support, or change in living situation.(73) [GG in Context Link 1]

[HH] Goals for care may be diagnostic, therapeutic, or rehabilitative in nature. Goals should be agreed upon by the patient (or guardian, as appropriate), and incorporate the patient's perspective on current problems, as well as the patient's specific values and preferences. Lack of progress toward goals should trigger a reassessment of the care plan (eg, determination as to accuracy of the admitting diagnosis, identification of barriers toward progress, incorporation of new problems that may have developed into the treatment plan, assessment of the appropriateness of initial management strategies), and lead to the modification of the plan as appropriate to promote optimal recovery.(10)(78) [HH in Context Link 1, 2]

[II] Evaluation of the lack of willingness to participate in treatment (eg, refusal to consent to or participate in treatment) should ascertain whether or not additional efforts to engage the patient in treatment are likely to be successful. It should also include a risk assessment (eg, whether or not the patient would represent a risk to self or others), as well as determine whether or not pursuing involuntary treatment is indicated.(1)(2)(3)(5) [II in Context Link 1, 2]

[JJ] Impairments in motivation to participate in treatment, limitations to engagement in care, and resistance to change are common in patients with mental health and substance use disorders, and may represent a feature of the disease process. A lack of motivation may indicate the need for more intensive services in order to help promote recovery or behavior change. Deficits in motivation or resistance to change should be addressed with therapies designed to enhance motivation to participate in treatment and work toward recovery (eg, motivational enhancement therapy). The level of motivation and goals/strategies designed to address motivation should be included in the care plan, and a lack of progress toward goals should trigger a reassessment of the care plan (eg, identification of barriers toward progress, incorporation of new problems which may have developed into the treatment plan, assessment of the appropriateness of initial management strategies), and lead to modification of the plan as appropriate to promote optimal recovery.(78)(79) [JJ in Context Link 1, 2]

[KK] A variety of factors may impact the ability of the patient to respond to treatment, including language, cognitive, and intellectual capacity, as well as the presence of any developmental conditions. Assessment of the ability to respond to planned interventions may help identify subgroups of patients more likely to respond to particular treatments, and help optimize (or modify) the care plan to increase the likelihood of successful recovery. It may also help determine if a specialized treatment setting (eg, dual-diagnosis program) or alternative level of care would be more appropriate to address the patient's needs. Patients who do not initially respond to scheduled therapeutic interventions may respond to modifications of the treatment plans or strategies designed to enhance ability. These include the addition or substitution of a new medication or psychosocial therapy, adjustment of the medication dose, increase in service intensity (some patients require higher levels of service intensity to provide reinforcement to enhance motivation and promote change), or decrease in service intensity (some patients may find higher levels of service intensity so restrictive as to be burdensome and hinder recovery).(1)(2)(3)(5)(32)(78)(79) [KK in Context Link 1, 2]

[LL] Partial hospital programs (PHPs) provide multidisciplinary behavioral care for 6 to 8 hours per day, 5 to 7 days per week (with patients going home each evening or weekend), and are staffed similarly to the day shift of an inpatient unit.(1)(2)(3)(4)(10)(70) [LL in Context Link 1]

[MM] A practice guideline for partial hospital and intensive outpatient care indicates that admission to a partial hospital program may be preferable to an intensive outpatient program if daily or near daily management or immediate intervention is necessary. Conditions that may require this intensity of monitoring include medication and comprehensive symptom management; observation and safety planning due to danger to self or others; lack of resiliency and need for repeated reinforcement; extreme mood swings, hopelessness, or isolation with inadequate or unavailable community supports; and substance use monitoring. Immediate intervention may be necessary for crisis situations (eg, volatile family situations) or when urgent behavioral activation is required (eg, rapid improvement is necessary to return the individual to vital role functioning).(10) [MM in Context Link 1, 2, 3, 4]

[NN] Partial hospital programs for children and adolescents often include a school component, while intensive outpatient programs more typically support academic needs through after-school programs.(4) [NN in Context Link 1, 2]

[OO] Composite score does not replace clinical judgment and is meant as a guide to assist in determining needed level of care. Severe or extreme risk of harm, serious or severe functional impairment, or major or severe comorbidity may independently necessitate a higher level of care.(1)(2)(3) [OO in Context Link 1, 2, 3]

[PP] Patients who are assessed to have significant potential for harm to self or others and require daily or near daily observation and safety planning may be more appropriately managed in a partial hospital program vs an intensive outpatient program.(10) [PP in Context Link 1, 2]

[QQ] An example of a medical condition appropriate for partial hospital care is asthma that is anticipated to require medication adjustment during the course of treatment.(1)(2)(3) [QQ in Context Link 1]

[RR] A practice guideline for partial hospital and intensive outpatient care indicates that admission to a partial hospital or an intensive outpatient program is inappropriate if the patient's needs are primarily social, custodial, recreational, or respite.(10) [RR in Context Link 1, 2]

[SS] Goals for care may be diagnostic, therapeutic, or rehabilitative in nature. Goals should be agreed upon by the patient (or guardian, as appropriate), and incorporate the patient's perspective on current problems, as well as the patient's specific values and preferences. Lack of progress toward goals should trigger a reassessment of the care plan (eg, determination as to the accuracy of the admitting diagnosis, identification of barriers toward progress, incorporation of new problems that may have developed into the treatment plan, assessment of the appropriateness of initial management strategies), and lead to the modification of the plan as appropriate to promote optimal recovery.(1)(2)(3)(10) [SS in Context Link 1, 2]

[TT] A practice guideline for partial hospital and intensive outpatient care indicates that inappropriate candidates for intensive outpatient and partial hospital programs include patients (or parents/guardians, as appropriate) who are uninterested or unable to engage in identifying goals for treatment, contract for behavioral change, or engage in a mutually agreed treatment plan.(10) [TT in Context Link 1, 2, 3]

[UU] Partial hospital program staff should verify that support persons are able to conduct any necessary monitoring of the patient's condition, provide instruction regarding the symptoms and behaviors to be monitored, and specify actions to be taken if the patient develops a current plan for suicide or serious harm.(10) [UU in Context Link 1]

[VV] A practice guideline for partial hospital and intensive outpatient care indicates that inappropriate candidates for intensive outpatient or partial hospital programs include patients with cognitive dysfunction or a developmental level that precludes the integration of newly learned information, skills, or behavioral change; patients with conditions (eg, social phobia, mania, anxiety, paranoia) assessed likely to be aggravated by care plans that emphasize the role of group treatment; and patients with lack of adherence (either due to unwillingness or lack of ability) to program expectations and personal responsibilities in a manner anticipated to negatively impact group functioning.(10) [VV in Context Link 1, 2]

[WW] Intensive outpatient programs (IOPs) typically provide 3 to 4 hours of psychosocial treatment 1 to 4 days per week (usually 6 to 12 hours of treatment per week), mostly by using a group format, and are appropriate for patients who need a type or frequency of psychosocial treatment that is not currently available in a standard outpatient setting but is available in an IOP.(1)(2)(3)(4)(10)(70) [WW in Context Link 1]

Context Link 1]

[XX] Examples of medical conditions appropriate for intensive outpatient care include asthma, hypertension, or diabetes that are being treated on stable medication regimens and not anticipated to require adjustment during the course of treatment.(1)(2)(3)(8) [XX in Context Link 1]

[YY] Biopsychosocial stressors may impact the level of care necessary to manage a psychiatric or behavioral condition, including the ability of the program to meet comprehensive patient needs, ensure treatment adherence, enhance motivation, or prevent relapse (ie, comorbidities, environmental factors, or other barriers may prevent effective treatment at a less intensive level of care than might otherwise be appropriate to the patient's condition). A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(28)(29)(30)(31) Biopsychosocial assessment factors should be incorporated into care planning, including planned treatment goals, and intensity and duration of interventions. Any identified deficits should be manageable by the program directly or through alternative arrangements.(1)(2)(3)(5) [YY in Context Link 1]

[ZZ] Outpatient care consists of pharmacologic and/or psychosocial treatment services provided in office or clinic settings (or through consultation to an assisted living, custodial care, or other special care program or facility) for managing care of behavioral health disorders. Patients at this level of care are generally seen at least twice a year, but visit frequencies are flexible and determined on the basis of individual patient needs.(1)(2)(3) [ZZ in Context Link 1]

[AAA] Outpatient behavioral health care may be carried out in conjunction with a variety of home-based services.(80) [AAA in Context Link 1]

[BBB] Examples of medical conditions appropriate for outpatient care include asthma on stable treatment regimen or HIV-positive status without symptoms.(1)(2)(3) [BBB in Context Link 1]

[CCC] Day treatment consists of a community-based mix of psychosocial treatment (including individual, family, and group-based psychotherapy), educational, and recreational activities for patients with behavioral health conditions associated with functional impairment (eg, inability to maintain full-time engagement in work, school, or home environment as appropriate). This level of care typically is monitored by staff whose licensure and credentials (eg, nurses, social workers) is allowed for this role by local statutes and is clinically appropriate to patient needs.(4)(82)(83)(84) Day treatment is designed to address issues that are chronic in nature, rather than acute exacerbations or urgent clinical issues; services tend to overlap with regular school or work schedules, and typically are of longer duration than intensive outpatient or partial hospital programs (eg, an adolescent in day treatment may be enrolled in a program that lasts for the entire school year).(4)(82)(83)(84) [CCC in Context Link 1]

[DDD] For some patients, day treatment programs provide a period of transition from a more intensive level of care and assist the patient in reintegrating into the community, as enrollment in day treatment following hospitalization has been associated with a decrease in the risk of rehospitalization.(4)(85) While patients for whom day treatment is indicated do not require the intensity of services available in an intensive outpatient or partial hospital program, some day treatment programs provide diagnostic, medical, psychiatric, or other adjunctive treatment modalities, either directly or through arrangements made by the program. These services may be provided over an extended period of time.(82) [DDD in Context Link 1]

[EEE] **Substance-related disorders:** Specialty centers providing outpatient treatment for substance use disorders commonly do so in day treatment centers.(6) Day treatment is appropriate to support abstinence and decrease the risk of relapse, including the promotion of strategies for the management of addiction symptoms (cravings) that pose a risk for relapse but do not require the level of support that would be provided in an intensive outpatient program.(4)(5)(6)(86) Patients appropriate for day treatment may require motivational enhancement and monitoring strategies to address barriers to change (despite at least some readiness for recovery) to help support engagement in treatment and promote continued progress through the stages of change, but do not require the level of support that would be provided in an intensive outpatient program.(5)(6)(10) Patients with substance use disorders who are appropriate for day treatment are generally resilient and require only minimal support to manage any deficits in the environment.(1)(2)(3)(5) Examples of mild environmental stressors include stress due to normal transition (eg, new school, new job), minor interpersonal loss (eg, loss of peer relationship due to move), transient but significant illness (eg, fractured bone), somewhat inadequate material resources, performance expectations that create discomfort, or potential for exposure to substance use.(1)(2)(3)(5)(6) [EEE in Context Link 1]

[FFF] **Eating disorders:** Patients appropriate for day treatment for eating disorders do not require the level of monitoring that would be provided in an intensive outpatient or partial hospital program (staff supervision of meals is not required), and any supervision of meals that is determined to be clinically appropriate may be provided by family, caregivers, or other supports.(10)(78) [FFF in Context Link 1]

[GGG] Determination of severity may be based on symptom severity alone (including intensity or frequency of symptoms and the extent to which they interfere with functioning), or may be due to other factors (eg, comorbid medical illness, developmental condition, cognitive impairment, substance use disorder, or other factors that contribute to destabilization or decreased ability to cope with the underlying behavioral health disorder). If the comorbidity affects the level of care appropriate to meet the patient's behavioral health needs, the manner in which it impacts the behavioral health condition (and how the comorbidity will be managed at the proposed level of care) should be documented in order to optimize comprehensive patient care.(1)(2)(3)(5)(7) [GGG in Context Link 1]

[HHH] Assessment for comorbid conditions should take place at admission evaluation, and any identified conditions should be manageable by the program (either directly or through alternative arrangements). Comorbidities requiring more active management or that currently distract the patient from therapy or have been associated with challenges in treatment follow-through in the past should prompt consideration of a program designed to provide treatment for both disorders (eg, a dual-diagnosis program).(1)(2)(3)(5)(7)(15) [HHH in Context Link 1]

[III] The diagnosis and severity of depressive disorders may be confirmed and documented by the use of standardized rating scales that reliably measure depressive symptoms. For adults, these scales include the Beck Depression Inventory (BDI), the Hamilton Depression Rating Scale (HDRS), the Patient Health Questionnaire-9 (PHQ-9) in the primary care setting, the Montgomery-Asberg Depression Rating Scale (MADRS), and the Geriatric Depression Scale (GDS) for older adults.(26)(87)(88) For children and adolescents, these include the BDI, the Children's Depression Inventory (CDI), and the PHQ-9 modified for adolescents (PHQ-A).(21)(22)(23) As an example, the PHQ-9 and PHQ-A ask the patient to describe the frequency of 9 symptoms over the past 2 weeks on a scale of 0 to 3 (0 = not at all, 1 = several days, 2 = more than half the days, 3 = nearly every day), and depression severity is scored as follows: 0 to 4 none, 5 to 9 mild, 10 to 14 moderate, 15 to 19 moderately severe, and 20 to 27 severe.(24) In addition to screening, rating scales are often administered throughout treatment to measure changes in symptoms. An approximate 50% decrease in scores on the BDI, HDRS, PHQ-9/PHQ-A, or the MADRS has been found to be clinically significant when reassessing an individual for interval improvement.(21)(25)(26) [III in Context Link 1]

[JJJ] Patients with recent substantial increases in signs and symptoms related to the underlying diagnosis or recent deterioration in function should be considered for a program with a greater level of service intensity (eg, an intensive outpatient or partial hospital program or other higher level of care).(82) [JJJ in Context Link 1]

[KKK] Assessment of the likelihood of benefit at the proposed level of care may help identify subgroups of patients more likely to respond to particular treatments and help optimize the care plan to increase the likelihood of successful recovery. It may also help determine if a specialized treatment setting (eg, dual-diagnosis program) or a different level of service intensity would be more appropriate to address the patient's needs.(1)(2)(3)(5)(6)(78) [KKK in Context Link 1]

[LLL] Evidence is inconclusive for animal-assisted interventions for autism spectrum disorder and mental health conditions based on heterogeneity in interventions, methodological weaknesses, small sample sizes, and lack of randomization within studies.(89)(90) [LLL in Context Link 1]

Definitions

Anxiety (adult)

- Multiple instruments are available to assess anxiety severity. The General Anxiety Disorder-7 (GAD-7) asks the patient to describe the frequency of 7 symptoms over the past 2 weeks on a scale of 0 to 3 (0 = not at all, 1 = several days, 2 = more than half the days, 3 = nearly every day). A score of 5 to 9 suggests mild anxiety; 10 to 14, moderate anxiety; and 15 or greater, severe anxiety.(1) Examples of anxiety symptoms include(2):
 - Feeling nervous, anxious, or on edge
 - Not being able to stop or control worrying
 - Worrying too much about different things
 - Trouble relaxing
 - Being so restless that it is hard to sit still
 - Becoming easily annoyed or irritable
 - Feeling afraid, as if something awful might happen

References

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2. Anxiety disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:215-261.

Anxiety (child or adolescent)

- Multiple instruments are available to assess anxiety severity, including the Multidimensional Anxiety Scale for Children-2nd Edition (MASC 2), the Screen for Child Anxiety-Related Emotional Disorders (SCARED), and the Spence Children's Anxiety Scale (SCAS).(1)(2)(3)(4)(5) Examples of anxiety symptoms include(6):
 - Nervousness
 - Feeling on edge
 - Inability to stop worrying or excessive worrying
 - Being told by others that one worries too much
 - Trouble relaxing

- Inability to sit still due to restlessness
- Easily irritable or annoyed
- Feeling afraid that something awful might happen
- Feeling frightened for no reason
- Feeling afraid to be alone in the house
- Feeling scared to go to school

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Command auditory hallucinations

- An auditory hallucination is a "false perception of sound." This is the most common type of hallucination in psychiatric disorders and typically consists of voices but also may consist of any type of sound. A command auditory hallucination is a false perception of a voice giving orders to the patient that they may feel compelled to obey. Likely adherence to a command auditory hallucination is indicated by **1 or more** of the following(1)(2)(3)(4):
 - Perceived familiar or benevolent source
 - No belief by patient in their ability to control the voices
 - History of frequent response to such hallucinations with unquestioning obedience

References

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4. Clementz BA, Keshavan MS, Pearlson GD, Sweeney JA, Tamminga CA. Phenotypes of psychosis. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1512-1519.

Current plan

- Determining the existence of current plan for suicide, homicide, or serious harm to self or another involves clinical judgment regarding multiple factors. Higher risk may be indicated by a combination of factors including explicit or implicit evidence of **1 or more** of the following(1)(2):
 - Identification and/or availability of severe or lethal method (eg, using a gun)
 - Preparatory acts (eg, writing suicide note)
 - Intention that act will have severe or fatal outcome evidenced by planning where and when action will be taken (eg, act unlikely to be discovered and likely to be fatal)

References

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2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.

Danger to others for adult

- Danger to others for adult is present due to **1 or more** of the following(1)(2)(3):
 - Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another
 - Patient has persistent thoughts of homicide or serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[A]:
 - Necessary behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Patient support system inadequate
 - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight; moderate to moderately severe Mania (adult) or unreliability
 - Indication or report of significant physical or sexual aggression with Impaired impulse control, judgment, or insight that significantly endangers another
 - Current homicidal ideation with either clearly expressed intentions or past history of carrying out such behavior

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.
3. The Assessment and Management of Suicide Risk Work Group. VA/DoD Clinical Practice Guideline for the Assessment and Management of Patients at Risk for Suicide. VA/DoD Evidence Based Practice [Internet] U.S. Department of Veteran Affairs and the Department of Defense. Version 2.0; 2019 Accessed at: <https://www.healthquality.va.gov/guidelines/>. [accessed 2023 May 15]

Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(2)

Danger to others for child or adolescent

- Danger to others for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another
 - Patient has persistent thoughts of homicide or serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[A]:
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight; moderate to moderately severe Mania (child or adolescent) or unreliability
 - Indication or report of significant physical or sexual aggression with Impaired impulse control, judgment, or insight that significantly endangers another
 - Current homicidal ideation with either clearly expressed intentions or past history of carrying out such behavior

References

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Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(7)

Danger to self for adult

- Danger to self for adult is present due to **1 or more** of the following(1)(2)(3):
 - Command auditory hallucinations contributing to risk for suicide or serious Harm to self
 - Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level of care as indicated by **1 or more** of the following(A):
 - Necessary behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Patient support system inadequate
 - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight; moderate to moderately severe Mania (adult) or unreliability
 - Indication or report of significant physical or sexual risky behavior with Impaired impulse control, judgment, or insight that significantly endangers self

References

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Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(2)

Danger to self for child or adolescent

- Danger to self for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Command auditory hallucinations contributing to risk for suicide or serious Harm to self(4)
 - Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level of care as indicated by **1 or more** of the following(A):
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as moderately severe to severely Impaired impulse control, judgment, or insight; moderate to moderately severe Mania (child or adolescent) or unreliability
 - Indication or report of significant physical or sexual risky behavior with Impaired impulse control, judgment, or insight that significantly endangers self

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Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(4)

Delusions (adult)

- Delusions are fixed beliefs that are not amenable to change in light of conflicting evidence. Common themes include persecutory, grandiose, erotomanic, nihilistic, and somatic. Delusions are considered bizarre if they are clearly implausible, not understandable to peers of the same culture, and do not derive from ordinary life experiences. Thought withdrawal, thought insertion, and delusions of control are examples of bizarre delusions.(1) Delusion severity may be described as(2)(3)(4):
 - Minimal (subtle symptoms that may rarely bother the patient, and are not associated with pressure to act on beliefs or interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, are associated with little pressure to act on beliefs, and contribute to **Mild** dysfunction in daily living for adult)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), are associated with little pressure to act on beliefs, and contribute to **Mild to moderate** dysfunction in daily living for adult)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), are associated with pressure to act on beliefs, and contribute to **Moderate** dysfunction in daily living for adult)
 - Moderately severe (symptoms may be bothersome to the patient, are associated with pressure to act on beliefs, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for adult, but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), are associated with strong pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for adult (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, are associated with intense pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for adult (condition usually requires close supervision))

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Delusions (child or adolescent)

- Delusions are fixed beliefs that are not amenable to change in light of conflicting evidence. Common themes include persecutory, grandiose, erotomanic, nihilistic, and somatic. Delusions are considered bizarre if they are clearly implausible, not understandable to peers of the same culture, and do not derive from ordinary life experiences. Thought withdrawal, thought insertion, and delusions of control are

examples of bizarre delusions. Instruments are available for assessing severity of symptoms in young people, including the instrument for positive and negative symptoms in severely disturbed children and adolescents (Kiddie-PANSS). Delusion severity may be described as(1)(2):

- Minimal (subtle symptoms that may rarely bother the patient, and are not associated with pressure to act on beliefs or interference with daily functioning (may represent extreme end of normal range))
- Mild (symptoms may not be very bothersome to the patient, are only occasionally present, are associated with little pressure to act on beliefs, and contribute to **Mild** dysfunction in daily living for child or adolescent)
- Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), are associated with little pressure to act on beliefs, and contribute to **Mild to moderate** dysfunction in daily living for child or adolescent)
- Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), are associated with pressure to act on beliefs, and contribute to **Moderate** dysfunction in daily living for child or adolescent)
- Moderately severe (symptoms may be bothersome to the patient, are associated with pressure to act on beliefs, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for child or adolescent, but are not all-consuming and usually can be contained at will)
- Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), are associated with strong pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for child or adolescent (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
- Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, are associated with intense pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for child or adolescent (condition usually requires close supervision))

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1. Schizophrenia spectrum and other psychotic disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:101-138.
2. Fields JH, et al. Assessing positive and negative symptoms in children and adolescents. American Journal of Psychiatry 1994;151(2):249-53.

Depressive symptoms (adult)

- Multiple instruments are available to assess depression severity and may be used to measure changes in depressive symptoms throughout treatment. For example, the Patient Health Questionnaire-9 (PHQ-9) for adults, used in the primary care setting, assesses how frequently the patient experienced the following symptoms over the past 2 weeks[A](1)(2)(3)(4)(5):
 - Interest in activities
 - Depressed mood/hopelessness
 - Disturbed sleep
 - Decreased energy
 - Change in appetite
 - Self-loathing
 - Difficulty concentrating
 - Psychomotor retardation
 - Suicidal thoughts

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Footnotes

- A. The diagnosis and severity of depressive disorders may be confirmed and documented by the use of standardized rating scales that reliably measure depressive symptoms in adults, including the Beck Depression Inventory (BDI), the Hamilton Depression Rating Scale (HDRS), the Patient Health Questionnaire-9 (PHQ-9) in the primary care setting, the Montgomery-Asberg Depression Rating Scale (MADRS), and the Geriatric Depression Scale (GDS) for older adults.(1)(2)(3) As an example, the PHQ-9 asks the patient to describe the frequency of 9 symptoms over the past 2 weeks on a scale of 0 to 3 (0 = not at all, 1 = several days, 2 = more than half the days, 3 = nearly every day), and depression severity is scored as follows: 0 to 4 none, 5 to 9 mild, 10 to 14 moderate, 15 to 19 moderately severe, and 20 to 27 severe.(4) In addition to screening, rating scales are often administered throughout treatment to measure changes in symptoms. An approximate 50% decrease in scores on the BDI, HDRS, PHQ-9, and/or MADRS has been found to be clinically significant when reassessing an individual for interval improvement.(2)(5)

Depressive symptoms (child or adolescent)

- Multiple instruments are available to assess depression severity and may be used to measure changes in depressive symptoms throughout treatment. For example, the Patient Health Questionnaire-9 (PHQ-9) modified for adolescents (PHQ-A), used in the primary care setting, assesses how frequently the patient experienced symptoms over the past 2 weeks. Of note, the PHQ-A has been validated for use with adolescents, but has not been validated for use with children younger than 11 years of age. Symptoms assessed includeA](1)(2)(3)(4)(5)(6)(7)(8):
 - Interest in activities
 - Depressed mood/hopelessness
 - Disturbed sleep
 - Decreased energy
 - Change in appetite
 - Self-loathing
 - Difficulty concentrating
 - Psychomotor retardation
 - Suicidal thoughts

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7. Lewandowski RE, et al. Screening for and diagnosis of depression among adolescents in a large health maintenance organization. Psychiatric Services 2016;67(6):636-41. DOI: 10.1176/appi.ps.201400465.
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Footnotes

- A. The diagnosis and severity of depressive disorders may be confirmed and documented by the use of standardized rating scales that reliably measure depressive symptoms in children and adolescents, including the Beck Depression Inventory (BDI), the Children's Depression Inventory (CDI), and the Patient Health Questionnaire-9 (PHQ-9) modified for adolescents (PHQ-A) in the primary care setting. (1)(2)(3) As an example, the PHQ-9 asks the patient to describe the frequency of 9 symptoms over the past 2 weeks on a scale of 0 to 3 (0 = not at all, 1 = several days, 2 = more than half the days, 3 = nearly every day), and depression severity is scored as follows: 0 to 4 none, 5 to 9 mild, 10 to 14 moderate, 15 to 19 moderately severe, and 20 to 27 severe.(4) In addition to screening, rating scales are often administered throughout treatment to measure changes in symptoms. An approximate 50% decrease in scores on the BDI and PHQ-A has been found to be clinically significant when reassessing an individual for interval improvement.(1)(5)(6)

Hallucinations (adult)

- Hallucinations are perceptions that occur without an external stimulus (eg, hearing, seeing, tasting, smelling, or feeling things that are not present). They may occur in any sensory modality, but auditory hallucinations are the most common in schizophrenia and related disorders.(1) Hallucination severity may be described as(2)(3)(4):
 - Minimal (subtle symptoms that may rarely bother the patient, and are not associated with pressure to act on beliefs or interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, are associated with little pressure to act on beliefs, and contribute to **Mild** dysfunction in daily living for adult)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), are associated with little pressure to act on beliefs, and contribute to **Mild to moderate** dysfunction in daily living for adult)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), are associated with pressure to act on beliefs, and contribute to **Moderate** dysfunction in daily living for adult)
 - Moderately severe (symptoms may be bothersome to the patient, are associated with pressure to act on beliefs, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for adult, but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), are associated with strong pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for adult (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, are associated with intense pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for adult (condition usually requires close supervision))

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Hallucinations (child or adolescent)

- Hallucinations are perceptions that occur without an external stimulus (eg, hearing, seeing, tasting, smelling, or feeling things that are not present). They may occur in any sensory modality, but auditory hallucinations are the most common in schizophrenia and related disorders. Instruments are available for assessing the severity of symptoms in young people, including the instrument for positive and negative symptoms in severely disturbed children and adolescents (Kiddie-PANSS).(1)(2)

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Harm

- Harm to self or another is considered serious if it has a substantial likelihood of causing death, disability, or major disfigurement.(1)(2)(3)(4)(5)

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Hyperactivity or inattention (adult)

- Hyperactivity refers to increased movement, impulsive actions, a decreased attention span, and easy distractibility. Inattention refers to easy distractibility and deficits in the ability to focus or stay on task. Determining severity includes the number of hyperactive or inattentive symptoms present, the frequency and duration of symptom presentation, and the impact that symptoms have on function and development. Levels of severity may be described as(1)(2):
 - Minimal (subtle symptoms that may rarely bother the patient, and are not associated with interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, and contribute to **Mild** dysfunction in daily living for adult)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), and contribute to **Mild to moderate** dysfunction in daily living for adult)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), and contribute to **Moderate** dysfunction in daily living for adult)
 - Moderately severe (symptoms may be bothersome to the patient, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for adult, but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), and contribute to **Severe** dysfunction in daily living for adult (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, and contribute to **Severe** dysfunction in daily living for adult (condition usually requires close supervision))

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Hyperactivity or inattention (child or adolescent)

- Hyperactivity refers to increased movement, impulsive actions, a decreased attention span, and easy distractibility. Inattention refers to easy distractibility and deficits in the ability to focus or stay on task. Determining severity includes the number of hyperactive or inattentive symptoms present, the frequency and duration of symptom presentation, and the impact that symptoms have on function and development. Levels of severity may be described as(1):
 - Minimal (subtle symptoms that may rarely bother the patient, and are not associated with interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, and contribute to **Mild** dysfunction in daily living for child or adolescent)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), and contribute to **Mild to moderate** dysfunction in daily living for child or adolescent)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), and contribute to **Moderate** dysfunction in daily living for child or adolescent)
 - Moderately severe (symptoms may be bothersome to the patient, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for child or adolescent, but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), and contribute to **Severe** dysfunction in daily living for child or adolescent (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, and contribute to **Severe** dysfunction in daily living for child or adolescent (condition usually requires close supervision))

References

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Imminent danger to others for adult

- Imminent danger to others for adult is present due to **1 or more** of the following(1)(2)(3):
 - Imminent risk for recurrence of attempt to seriously Harm another is present, as indicated by **ALL** of the following:
 - There has been a recent attempt to seriously Harm another.
 - There has not been Sufficient relief of factors that precipitated the attempt.
 - Current plan for homicide or serious Harm to another is present.
 - Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another are present.
 - Patient has persistent thoughts of homicide or violent acts that likely could result in serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[A]:
 - Necessary behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Patient support system inadequate
 - Patient characteristics, such as extremely Impaired impulse control, judgment, or insight; severe Mania (adult), severe unreliability, or extreme agitation with desperation
 - Ruminative flooding; uncontrollable and overwhelming negative thoughts
 - Indication or report of repeated behavior, including physical or sexual aggression that is clearly injurious to others with history, plan or intent, and extremely Impaired impulse control, judgment, or insight

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Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(2)

Imminent danger to others for child or adolescent

- Imminent danger to others for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Imminent risk for recurrence of attempt to seriously Harm another is present, as indicated by **ALL** of the following:
 - There has been a recent attempt to seriously Harm another.
 - There has not been Sufficient relief of factors that precipitated the attempt.
 - Current plan for homicide or serious Harm to another is present.
 - Command auditory hallucinations or paranoid delusions contributing to risk for homicide or serious Harm to another are present.
 - Patient has persistent thoughts of homicide or violent acts that likely could result in serious Harm to another that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[A]:
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Severe conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as extremely Impaired impulse control, judgment, or insight; severe Mania (child or adolescent), severe unreliability, or extreme agitation with desperation
 - Ruminative flooding; uncontrollable and overwhelming negative thoughts
 - Indication or report of repeated behavior, including physical or sexual aggression that is clearly injurious to others with history, plan or intent, and extremely Impaired impulse control, judgment, or insight

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Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(7)

Imminent danger to self for adult

- Imminent danger to self for adult is present due to **1 or more** of the following(1)(2)(3):
 - Imminent risk for recurrence of Suicide attempt or act of serious Harm to self is present, as indicated by **ALL** of the following:
 - There has been a recent Suicide attempt or deliberate act of serious Harm to self.
 - There has not been Sufficient relief of the factors that precipitated attempt or act.
 - Current plan for suicide or serious Harm to self is present.
 - Command auditory hallucinations or paranoid delusions contributing to risk for suicide or serious Harm to self are present.
 - Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following[A]:
 - Necessary behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Patient support system inadequate
 - Patient characteristics, such as extremely Impaired impulse control, judgment, or insight; severe Mania (adult), severe unreliability, or extreme agitation with desperation
 - Ruminative flooding; uncontrollable and overwhelming negative thoughts
 - Frantic hopelessness; fatalistic conviction that life will not improve, along with oppressive sense of entrapment and doom

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Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(2)

Imminent danger to self for child or adolescent

- Imminent danger to self for child or adolescent is present due to **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Imminent risk for recurrence of Suicide attempt or act of serious Harm to self is present, as indicated by **ALL** of the following:
 - There has been a recent Suicide attempt or deliberate act of serious Harm to self.
 - There has not been Sufficient relief of the factors that precipitated attempt or act.
 - Current plan for suicide or serious Harm to self is present.
 - Command auditory hallucinations or paranoid delusions contributing to risk for suicide or serious Harm to self are present.
- Patient has persistent Thoughts of suicide or serious Harm to self that cannot be adequately monitored or treated at lower level of care, as indicated by **1 or more** of the following(A):
 - Necessary child or adolescent behavioral health care (such as required provider or lower level of care) not available or insufficient
 - Severe conflict in family environment or other inadequacy in patient support system
 - Patient characteristics, such as extremely Impaired impulse control, judgment, or insight; severe Mania (child or adolescent), severe unreliability, or extreme agitation with desperation
 - Ruminative flooding; uncontrollable and overwhelming negative thoughts
 - Frantic hopelessness; fatalistic conviction that life will not improve, along with oppressive sense of entrapment and doom

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Footnotes

- A. Patients with persistent thoughts of suicide, homicide, or serious harm to self or another may need frequent monitoring for progression to planning or intention to act on their thoughts. Such monitoring can be safely conducted at an outpatient level of care if providers and supports are adequately available and the patient can reliably participate in the monitoring process.(4)

Impaired impulse control, judgment, or insight

- Impaired impulse control describes a lack of control or ability to regulate actions related to inner urges. Impaired judgment and insight may include a lack of recognition of current or past symptoms, denial of need for treatment, or lack of recognition of consequences of actions. Levels of severity may be described as(1)(2)(3):
 - Minimal (subtle symptoms that may rarely bother the patient and are not associated with interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, and contribute to **Mild** dysfunction in daily living for adult or **Mild** dysfunction in daily living for child or adolescent)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), and contribute to **Mild to moderate** dysfunction in daily living for adult or **Mild to moderate** dysfunction in daily living for child or adolescent)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), and contribute to **Moderate** dysfunction in daily living for adult or **Moderate** dysfunction in daily living for child or adolescent)
 - Moderately severe (symptoms may be bothersome to the patient, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for adult or **Serious** dysfunction in daily living for child or adolescent but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), and contribute to **Severe** dysfunction in daily living for adult or **Severe** dysfunction in daily living for child or adolescent (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, and contribute to **Severe** dysfunction in daily living for adult or **Severe** dysfunction in daily living for child or adolescent (condition usually requires close supervision))

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Mania (adult)

- Mania symptoms may include elated/irritable mood, grandiosity, decreased need for sleep (eg, feeling rested after only a few hours of sleep), hyperverbal or rapid pressured speech, flight of ideas, racing thoughts, distractibility (ie, attention too easily drawn to unimportant or irrelevant external stimuli), increase in goal-directed activity (either socially, at work, or at school), hypersexuality, excessive energy, psychomotor agitation, or excessive involvement in pleasurable activities that have a high potential for negative consequences (eg, engaging in unrestrained buying sprees, sexual indiscretions, or foolish business investments). Symptoms do not have an alternative explanation (eg, intoxication, delirium, or other medical condition). Symptom severity may be described as(1)(2)(3)(4)(5)(6):
 - Mild (periods of somewhat elevated, expansive, or irritable mood, or other manic symptoms that are occasionally present)
 - Mild to moderate (periods of somewhat elevated, expansive, or irritable mood, or other manic symptoms that are present up to half the days of each week)
 - Moderate (periods of extensively elevated, expansive, or irritable mood or restlessness, or other manic symptoms that are present more than half the days of each week)
 - Moderately severe (frequent, but not daily, periods of extensively elevated, expansive, or irritable mood or restlessness, or other manic symptoms)
 - Severe (daily periods of extensively elevated, expansive, or irritable mood or restlessness, or other manic symptoms)

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Mania (child or adolescent)

- Mania symptoms may include elated/irritable mood, grandiosity, decreased need for sleep (eg, feeling rested after only a few hours of sleep), hyperverbal or rapid pressured speech, flight of ideas, racing thoughts, distractibility (ie, attention too easily drawn to unimportant or irrelevant external stimuli), increase in goal-directed activity (eg, socially or at school), hypersexuality, excessive energy, psychomotor agitation, or excessive involvement in pleasurable activities that have a high potential for negative consequences. Symptoms do not have an alternative explanation (eg, intoxication, delirium, or other medical condition).(1)(2)

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Mild dysfunction in daily living for adult

- Mild** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Some deterioration in interpersonal interactions (eg, increased conflict), but able to maintain some meaningful relationships

- Some recent minor disruptions in self-care
- Minor but consistent disruptions in social role functioning and meeting obligations, but still able to maintain these roles
- Self-neglect with at least some minor limitation in ability to provide for basic needs (by self or others) in maintenance care
- Other evidence of mild dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
3. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 36-Item Instrument Scoring Sheet, Simple Scoring Calculation [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
4. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 12-Item Instrument Scoring Sheet [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
5. Sheehan DV, Harnett-Sheehan K, Raj BA. The measurement of disability. International Clinical Psychopharmacology 1996;11 Suppl 3:89-95.
6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Mild dysfunction in daily living for child or adolescent

- **Mild** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Minor deterioration or episodic failure in relationships with peers, family, or adults (eg, defiance, lying, provocative behavior)
 - Self-care sporadically below usual or expected standards
 - Impairments in function characterized by ability to show improvements following episodes of deterioration
 - Diminished ability to assess consequences of own actions (eg, acts of severe property damage) that therapeutic intervention can remediate
 - Self-neglect with at least some minor limitation in ability to provide for basic needs (by self or others) in maintenance care
 - Other evidence of mild dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
2. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. International Classification of Functioning, Disability and Health. Children & Youth Version [Internet] World Health Organization. 2007 Accessed at: <https://www.who.int/classifications/icf/en/>. [accessed 2022 Apr 28]
5. Shaffer D, et al. A children's global assessment scale (CGAS). Archives of General Psychiatry 1983;40(11):1228-31.
6. Lundh A, Kowalski J, Sundberg CJ, Gumpert C, Landen M. Children's Global Assessment Scale (CGAS) in a naturalistic clinical setting: Inter-rater reliability and comparison with expert ratings. Psychiatry Research 2010;177(1-2):206-10. DOI: 10.1016/j.psychres.2010.02.006.
7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Mild to moderate dysfunction in daily living for adult

- **Mild to moderate** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Some recent deterioration in interpersonal interactions (eg, conflicted, withdrawn), but maintains control of impulsive or abusive behaviors and maintains some meaningful relationships
 - Some recent disruptions in self-care below usual or expected standards
 - Disturbance in vegetative status (eg, eating, sleeping habits) that does not pose serious threat to health
 - Some deterioration in social role functioning and meeting obligations (eg, avoiding responsibilities on some occasions), but still can maintain roles overall
 - Other evidence of mild to moderate dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
3. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 36-Item Instrument Scoring Sheet, Simple Scoring Calculation [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
4. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 12-Item Instrument Scoring Sheet [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
5. Sheehan DV, Harnett-Sheehan K, Raj BA. The measurement of disability. International Clinical Psychopharmacology 1996;11 Suppl 3:89-95.
6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Mild to moderate dysfunction in daily living for child or adolescent

- **Mild to moderate** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Some difficulty in relationships with peers, family, or adults (eg, defiance, lying), but without physical aggression
 - Self-care sometimes below usual or expected standards
 - Disturbance in vegetative status (eg, eating, sleeping habits) that does not pose serious threat to health
 - Deteriorating school behavior such that disruption in school is threatened (eg, threat of expulsion)
 - Diminished ability to assess consequences of own actions (eg, acts of severe property damage) that therapeutic intervention can remediate
 - Other evidence of mild to moderate dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
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3. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
4. International Classification of Functioning, Disability and Health. Children & Youth Version [Internet] World Health Organization. 2007 Accessed at: <https://www.who.int/classifications/icf/en/>. [accessed 2022 Apr 28]
5. Shaffer D, et al. A children's global assessment scale (CGAS). Archives of General Psychiatry 1983;40(11):1228-31.
6. Lundh A, Kowalski J, Sundberg CJ, Gumpert C, Landen M. Children's Global Assessment Scale (CGAS) in a naturalistic clinical setting: Inter-rater reliability and comparison with expert ratings. Psychiatry Research 2010;177(1-2):206-10. DOI: 10.1016/j.psychres.2010.02.006.
7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Moderate dysfunction in daily living for adult

- **Moderate** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Recent conflicted, withdrawn, or troubled relationships, but maintains control of impulsive, aggressive, or abusive behaviors
 - Self-care frequently below usual or expected standards
 - Significant disturbance in vegetative status (eg, eating, sleeping habits), but not posing serious threat to health
 - Significant deterioration in ability to fulfill responsibilities (eg, work, commitments to significant others), including neglect or avoidance of these responsibilities on some occasions
 - Ongoing and variably severe deficits in interpersonal relationships and ability to engage socially in constructive manner
 - Other evidence of moderate dysfunction

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
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4. WHO Disability Assessment Schedule 2.0 (WHODAS 2.0). 12-Item Instrument Scoring Sheet [Internet] World Health Organization. 2014 Nov Accessed at: <https://www.who.int/>. [accessed 2023 Aug 14]
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6. Sheehan Disability Scale (SDS). [Internet] Psychiatry & Behavioral Health Learning Network. 2019 Jan 28 Accessed at: <https://www.psychcongress.com/saundras-corner/scales-screeners/disability-scales/sheehan-disability-scale-sds>. [created 1983; accessed 2023 Aug 11]

Moderate dysfunction in daily living for child or adolescent

- **Moderate** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Troubled relationships with peers, family, or adults (eg, conflicted, withdrawn), but without physical aggression
 - Self-care frequently below usual or expected standards
 - Significant disturbance in vegetative status (eg, eating, sleeping habits), but not posing serious threat to health
 - Deteriorated school behavior such that disruption in school has occurred (eg, suspension, expulsion), and child is at risk for placement in alternative school setting or repeating grade
 - Diminished ability to assess consequences of own actions (eg, acts of severe property damage)
 - Other evidence of moderate dysfunction

References

1. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
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6. Lundh A, Kowalski J, Sundberg CJ, Gumpert C, Landen M. Children's Global Assessment Scale (CGAS) in a naturalistic clinical setting: Inter-rater reliability and comparison with expert ratings. Psychiatry Research 2010;177(1-2):206-10. DOI: 10.1016/j.psychres.2010.02.006.
7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Negative symptoms (adult)

- Negative symptoms represent thoughts, feelings, or behaviors that normally would be present, but are absent or diminished in relation to a psychiatric disorder. They may include apathy, anhedonia, poverty of speech, motor behavior that is diminished or signs or symptoms of psychomotor retardation.(1)(2)(3) Levels of severity may be described as:
 - Minimal (subtle symptoms that may rarely bother the patient, and are not associated with interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, and contribute to **Mild** dysfunction in daily living for adult)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), and contribute to **Mild to moderate** dysfunction in daily living for adult)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), and contribute to **Moderate** dysfunction in daily living for adult)
 - Moderately severe (symptoms may be bothersome to the patient, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for adult, but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), and contribute to **Severe** dysfunction in daily living for adult (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, and contribute to **Severe** dysfunction in daily living for adult (condition usually requires close supervision))

References

1. Kay SR, Fiszbein A, Opler LA. The positive and negative syndrome scale (PANSS) for schizophrenia. Schizophrenia Bulletin 1987;13(2):261-276.
2. The PANSS Institute. [Internet] The PANSS Institute. Accessed at: <http://www.panss.org/>. Updated 2023 [accessed 2023 Sep 12]
3. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.

Negative symptoms (child or adolescent)

- Negative symptoms represent thoughts, feelings, or behaviors that normally would be present, but are absent or diminished in relation to a psychiatric disorder. They may include apathy, anhedonia, poverty of speech, motor behavior that is diminished or signs or symptoms of psychomotor retardation. Instruments are available for assessing severity of symptoms in young people, including the instrument for positive and negative symptoms in severely disturbed children and adolescents (Kiddie-PANSS).(1)(2) Levels of severity may be described as:
 - Minimal (subtle symptoms that may rarely bother the patient, and are not associated with interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, and contribute to **Mild** dysfunction in daily living for child or adolescent)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), and contribute to **Mild to moderate** dysfunction in daily living for child or adolescent)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), and contribute to **Moderate** dysfunction in daily living for child or adolescent)
 - Moderately severe (symptoms may be bothersome to the patient, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for child or adolescent, but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), and contribute to **Severe** dysfunction in daily living for child or adolescent (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, and contribute to **Severe** dysfunction in daily living for child or adolescent (condition usually requires close supervision))

References

1. Schizophrenia spectrum and other psychotic disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:101-138.
2. Fields JH, et al. Assessing positive and negative symptoms in children and adolescents. American Journal of Psychiatry 1994;151(2):249-53.

No current plan

- Determining that no current plan exists for suicide, homicide, or serious harm to self or another involves clinical judgment regarding multiple factors. Lower risk may be indicated when **ALL** of the following are met(1)(2):
 - No evidence of identification and/or availability of severe or lethal method (eg, using a gun)
 - No evidence of preparatory acts (eg, writing suicide note)
 - No intention that an act that will have severe or fatal outcome (ie, no evidence of plan for where and when an action will be taken)

References

1. Sudak HS. Suicide treatment. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2618-2622.
2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.

Obsessions or compulsions (adult)

- Obsessions are recurrent and persistent thoughts, urges, or images that are intrusive and unwanted, which often cause marked anxiety or distress, and are associated with attempts to ignore, suppress, or neutralize these experiences.(1)(2) Compulsions are repetitive behaviors that the individual feels driven to perform in response to an obsession or according to rules that must be applied rigidly; the behaviors or mental acts are aimed at preventing or reducing anxiety or distress, or preventing some dreaded event or situation, but are not connected in a realistic way with what they are designed to neutralize or prevent (or are clearly excessive).(1)(2)

References

1. Obsessive-compulsive and related disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:263-294.
2. Goodman WK, et al. The Yale-Brown Obsessive Compulsive Scale. I. Development, use, and reliability. Archives of General Psychiatry 1989;46(11):1006-11.

Obsessions or compulsions (child or adolescent)

- Obsessions are recurrent and persistent thoughts, urges, or images that are intrusive and unwanted, which often cause marked anxiety or distress, and are associated with attempts to ignore, suppress, or neutralize these experiences.(1)(2) Compulsions are repetitive behaviors that the individual feels driven to perform in response to an obsession or according to rules that must be applied rigidly; the behaviors or mental acts are aimed at preventing or reducing anxiety or distress, or preventing some dreaded event or situation, but are not connected in a realistic way with what they are designed to neutralize or prevent (or are clearly excessive). Young children may not be able to articulate the aims of these behaviors or mental acts.(1)(2)(3)

References

1. Obsessive-compulsive and related disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:263-294.
2. Goodman WK, et al. The Yale-Brown Obsessive Compulsive Scale. I. Development, use, and reliability. Archives of General Psychiatry 1989;46(11):1006-11.
3. Lopez-Pina JA, et al. Reliability generalization study of the Yale-Brown Obsessive-Compulsive Scale for children and adolescents. Journal of Personality Assessment 2015;97(1):42-54. DOI: 10.1080/00223891.2014.930470.

Other emotional or behavioral disturbance

- Emotional or behavioral disturbances impacting symptoms or function, including **1 or more** of the following^A(3)(4)(5):
 - Externalizing symptoms (eg, angry outbursts, physical or verbal aggression, oppositional defiant traits, agitation, anxiety, or other disruptive behaviors)
 - Internalizing symptoms (eg, sulking, rumination, anhedonia, dysphoria, apathy)
 - Evidence of severely diminished ability to assess consequences of own actions (eg, acts of severe property damage)
 - High levels of family conflict or interpersonal conflict
 - Escalating relapse behaviors
 - Other emotional or behavioral disturbance relevant to clinical presentation

References

1. Kuhn TM, Ebert JS, Gracey KA, Chapman GL, Epstein RA. Evidence-based interventions for adolescents with disruptive behaviors in school-based settings. Child and Adolescent Psychiatric Clinics of North America 2015;24(2):305-17. DOI: 10.1016/j.chc.2014.11.005.
2. Kennedy A, Cloutier P, Glennie JE, Gray C. Establishing best practice in pediatric emergency mental health: a prospective study examining clinical characteristics. Pediatric Emergency Care 2009;25(6):380-6. DOI: 10.1097/PEC.0b013e3181a79223.
3. Chun TH, Katz ER, Duffy SJ, Gerson RS. Challenges of managing pediatric mental health crises in the emergency department. Child and Adolescent Psychiatric Clinics of North America 2015;24(1):21-40. DOI: 10.1016/j.chc.2014.09.003.
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5. Level of care placement. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:174-306.

Footnotes

- A. Symptoms may be circumscribed (eg, attention-seeking behaviors) or may be seen in the context of one or more major psychiatric disorders, including depression; anxiety disorders; obsessive-compulsive disorder; posttraumatic stress disorder; eating disorders; attention-deficit hyperactivity disorders; bipolar disorders; schizophrenia; personality disorders; autism spectrum disorder and other disruptive, impulse-control, and conduct disorders.(1)(2)(3)

Positive symptoms (adult)

- Positive symptoms are feelings, thoughts, perceptions, or behaviors that usually are not present. They may include hallucinations, delusions, or other symptoms that are abnormal or bizarre. Levels of severity may be described as(1)(2)(3)(4):
 - Minimal (subtle symptoms that may rarely bother the patient, and are not associated with pressure to act on beliefs or interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, are associated with little pressure to act on beliefs, and contribute to **Mild** dysfunction in daily living for adult)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), are associated with little pressure to act on beliefs, and contribute to **Mild to moderate** dysfunction in daily living for adult)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), are associated with pressure to act on beliefs, and contribute to **Moderate** dysfunction in daily living for adult)
 - Moderately severe (symptoms may be bothersome to the patient, are associated with pressure to act on beliefs, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for adult, but are not all-consuming and usually can be contained at will)
 - Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), are associated with strong pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for adult (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
 - Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, are associated with intense pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for adult (condition usually requires close supervision))

References

1. Kay SR, Fiszbein A, Opler LA. The positive and negative syndrome scale (PANSS) for schizophrenia. Schizophrenia Bulletin 1987;13(2):261-276.
2. The PANSS Institute. [Internet] The PANSS Institute. Accessed at: <http://www.panss.org/>. Updated 2023 [accessed 2023 Sep 12]
3. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857.
4. Schizophrenia spectrum and other psychotic disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:101-138.

Positive symptoms (child or adolescent)

- Positive symptoms are feelings, thoughts, perceptions, or behaviors that usually are not present. They may include hallucinations, delusions, or other symptoms that are abnormal or bizarre. Instruments are available for assessing severity of symptoms in young people, including the instrument for positive and negative symptoms in children and adolescents (Kiddie-PANSS).(1)(2) Levels of severity may be described as:
 - Minimal (subtle symptoms that may rarely bother the patient, and are not associated with pressure to act on beliefs or interference with daily functioning (may represent extreme end of normal range))
 - Mild (symptoms may not be very bothersome to the patient, are only occasionally present, are associated with little pressure to act on beliefs, and contribute to **Mild** dysfunction in daily living for child or adolescent)
 - Mild to moderate (symptoms may not be very bothersome to the patient, are established (eg, present up to half of each week), are associated with little pressure to act on beliefs, and contribute to **Mild to moderate** dysfunction in daily living for child or adolescent)
 - Moderate (symptoms may be somewhat bothersome to the patient, are clearly established (eg, present more than half of each week, but not daily or near daily), are associated with pressure to act on beliefs, and contribute to **Moderate** dysfunction in daily living for child or adolescent)

- Moderately severe (symptoms may be bothersome to the patient, are associated with pressure to act on beliefs, are present more than half of each week (but not daily or near daily), and contribute to **Serious** dysfunction in daily living for child or adolescent, but are not all-consuming and usually can be contained at will)
- Severe (symptoms may be very bothersome to the patient, are present daily (or near daily), are associated with strong pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for child or adolescent (eg, symptoms may be all-consuming or cannot be contained at will, and often require close supervision))
- Extreme (symptoms may be highly disturbing to the patient, are present on an almost constant basis, are associated with intense pressure to act on beliefs, and contribute to **Severe** dysfunction in daily living for child or adolescent (condition usually requires close supervision))

References

1. Schizophrenia spectrum and other psychotic disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:101-138.
2. Fields JH, et al. Assessing positive and negative symptoms in children and adolescents. American Journal of Psychiatry 1994;151(2):249-53.

Psychiatric symptoms (adult)

- Psychiatric symptoms, including **1 or more** of the following(1):
 - Hallucinations (adult), Delusions (adult), or other Positive symptoms (adult)
 - Negative symptoms (adult), including social or emotional withdrawal or lack of self-initiated behavior or movement
 - Mania (adult)
 - Depressive symptoms (adult)
 - Anxiety (adult)
 - Hyperactivity or inattention (adult)
 - Obsessions or compulsions (adult)
 - Personality disorders/maladaptive personality traits
 - Other psychiatric symptoms related to underlying psychiatric disorder or underlying comorbid condition having a negative impact on primary psychiatric disorder

References

1. Matorin AA, Shah AA, Ruiz P. Clinical manifestations of psychiatric disorders. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1114-1150.

Psychiatric symptoms (child or adolescent)

- Psychiatric symptoms, including **1 or more** of the following(1):
 - Hallucinations (child or adolescent), Delusions (child or adolescent), or other Positive symptoms (child or adolescent)
 - Negative symptoms (child or adolescent), including social or emotional withdrawal or lack of self-initiated behavior or movement
 - Mania (child or adolescent)
 - Depressive symptoms (child or adolescent)
 - Anxiety (child or adolescent)
 - Hyperactivity or inattention (child or adolescent)
 - Obsessions or compulsions (child or adolescent)
 - Personality disorders/maladaptive personality traits
 - Other psychiatric symptoms related to underlying psychiatric disorder or underlying comorbid condition having a negative impact on primary psychiatric disorder

References

1. Matorin AA, Shah AA, Ruiz P. Clinical manifestations of psychiatric disorders. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1114-1150.

Psychiatric, behavioral, or other comorbid conditions for adult

- Psychiatric, behavioral, or other comorbid conditions for adult are appropriate for management at proposed level of care, as indicated by **1 or more** of the following^[A]:
 - Psychiatric symptoms (adult)
 - Comorbid biomedical or developmental condition^{[B][C]}
 - Comorbid substance use disorder^{[D][E]}
 - Cognitive or memory impairment^[F]
 - Impaired impulse control, judgment, or insight
 - Other emotional or behavioral disturbance

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
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Footnotes

- A. Determination of severity may be based on symptom severity alone (including intensity or frequency of symptoms and the extent to which they interfere with functioning), or may be due to other factors (eg, comorbid medical illness, developmental condition, cognitive impairment, substance use disorder, or other factors that contribute to destabilization or decreased ability to cope with the underlying behavioral health disorder). If the comorbidity affects the level of care appropriate to meet the patient's behavioral health needs, the manner in which it impacts the behavioral health condition (and how the comorbidity will be managed/treated at the proposed level of care) should be documented in order to optimize comprehensive patient care. Assessment for comorbid conditions should take place at admission evaluation, and any identified conditions should be manageable/treatable by the program (either directly or through alternative arrangements). Co-occurring mental health and substance use conditions and co-occurring medical and behavioral health conditions are considered an expectation, not an exception, in all programs at all psychiatric and addiction service levels of care. Integrated care, with recovery-oriented and "co-occurring capable" services, is increasingly expected to be included in the design of programs at all levels of care and in all settings. In programs without integrated care, comorbidities that require more active management/treatment or that currently distract the patient from treatment or have been associated with challenges in treatment follow-through in the past should prompt consideration of a program designed to provide treatment for both disorders (eg, a dual-diagnosis program). (1)(2)(3)(4)(5)(6)(7)(8)
- B. A significant comorbid biomedical or developmental condition has the potential to prolong the course of illness, or necessitate admission to a higher level of care or closer monitoring of patients requiring psychiatric care. (1)(3)(4)(5)(9)
- C. Assessment and treatment of co-occurring medical and/or developmental conditions through services and treatment settings capable of rendering integrated care is recommended. (1)(2)(3)(4)(5)(10) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health. (11)(12)(13)(14) The level that comorbid medical and/or developmental conditions are present may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care). (1)(3)(4)(5)(15)
- D. A significant comorbid substance use disorder (either reflecting comorbidity in a patient whose primary diagnosis is a psychiatric disorder, such as an affective disorder, or is present in a patient with a polysubstance use disorder) has the potential to impact the appropriate level of care in which the underlying condition is managed, or require consideration of a program with structures in place to treat multiple conditions, such as a dual-diagnosis program. The risk for problems with substance use is increased with the presence of many types of behavioral disturbance (eg, impulse control disorders, or self-medication of depression or anxiety symptoms). The severity of the substance use disorder as it relates to the underlying psychiatric condition includes the frequency and chronicity of use, ability to control use, intensity of other symptoms (eg, cravings, withdrawal), or other negative impact on the underlying condition. (1)(3)(4)(5)(7)(8)
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- F. Intake and assessment should include a mental status examination and memory assessment. If cognitive impairments are identified, they may require modifying the intensity or pace of planned interventions to allow the patient to participate in the program, or using an enhanced program designed to address co-occurring cognitive disorders. (8)(16)(17)(18)(19)(20)

Psychiatric, behavioral, or other comorbid conditions for child or adolescent

- Psychiatric, behavioral, or other comorbid conditions for child or adolescent are appropriate for management at proposed level of care, as indicated by **1 or more** of the following[A]:
 - Psychiatric symptoms (child or adolescent)
 - Comorbid biomedical or developmental condition[B][C]
 - Comorbid substance use disorder[D][E]
 - Cognitive or memory impairment[F]
 - Impaired impulse control, judgment, or insight
 - Other emotional or behavioral disturbance

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- A. Determination of severity may be based on symptom severity alone (including intensity or frequency of symptoms and the extent to which they interfere with functioning), or may be due to other factors (eg, comorbid medical illness, developmental condition, cognitive impairment, substance use disorder, or other factors that contribute to destabilization or decreased ability to cope with the underlying behavioral health disorder). If the comorbidity affects the level of care appropriate to meet the patient's behavioral health needs, the manner in which it impacts the behavioral health condition (and how the comorbidity will be managed/treated at the proposed level of care) should be documented in order to optimize comprehensive patient care. Assessment for comorbid conditions should take place at admission evaluation, and any identified conditions should be manageable/treatable by the program (either directly or through alternative arrangements). Co-occurring mental health and substance use conditions and co-occurring medical and behavioral health conditions are considered an expectation, not an exception, in all programs at all psychiatric and addiction service levels of care. Integrated care, with recovery-oriented and "co-occurring capable" services, is increasingly expected to be included in the design of programs at all levels of care and in all settings. In programs without integrated care, comorbidities that require more active management/treatment or that currently distract the patient from treatment or have been associated with challenges in treatment follow-through in the past should prompt consideration of a program designed to provide treatment for both disorders (eg, a dual-diagnosis program). (1)(2)(3)(4)(5)(6)(7)(8)
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multiple conditions, such as a dual-diagnosis program. The risk for problems with substance use is increased with the presence of many types of behavioral disturbance (eg, impulse control disorders, or self-medication of depression or anxiety symptoms). The severity of the substance use disorder as it relates to the underlying psychiatric condition includes the frequency and chronicity of use, ability to control use, intensity of other symptoms (eg, cravings, withdrawal), or other negative impact on the underlying condition.(1)(3)(4)(5)(7)(8)

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Serious dysfunction in daily living for adult

- **Serious** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Serious deterioration in quality of interpersonal interactions (eg, impulsive or abusive behaviors)
 - Significant withdrawal and avoidance of almost all social interaction
 - Consistent failure to achieve self-care (eg, personal hygiene, appearance) to usual standards
 - Serious disturbance in vegetative status (eg, weight change, sleep disruption) threatening physical function
 - Inability to perform close to usual standards in adult obligations (eg, work, parenting) and neglecting these responsibilities frequently or over extended period of time
 - Other evidence of serious dysfunction

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Serious dysfunction in daily living for child or adolescent

- **Serious** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Serious deterioration in interpersonal interactions (eg, impulsive or abusive behaviors)
 - Significant withdrawal and avoidance of almost all social interaction
 - Consistent failure to achieve self-care as appropriate to age or developmental level
 - Serious disturbance in vegetative status (eg, weight change, sleep disruption) threatening physical function
 - Inability to perform adequately in school (including specialized setting) due to disruptive or aggressive behavior
 - Severely diminished ability to assess consequences of own actions (eg, acts of severe property damage)
 - Other evidence of serious dysfunction

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Severe dysfunction in daily living for adult

- **Severe** dysfunction in daily living for adult is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Incapacitation because of grave disability (eg, severe regression with inability to provide for self)
 - Extreme deterioration in social interactions (eg, threatening behaviors with little or no provocation)
 - Complete withdrawal from all social interactions
 - Complete neglect of self-care with associated impairment in physical status
 - Extreme disruption in vegetative function (eg, life-sustaining functions such as eating)
 - Complete inability to maintain any appropriate aspect of personal responsibility in any adult roles (eg, occupational, parental)
 - Other evidence of severe dysfunction

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Severe dysfunction in daily living for child or adolescent

- **Severe** dysfunction in daily living for child or adolescent is present, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6)(7):
 - Incapacitation because of grave disability (eg, severe regression with inability to provide for self)
 - Extreme deterioration in interactions with peers, adults, or family (eg, assaultive behaviors with no provocation, high levels of family conflict)
 - Complete withdrawal from all social interactions
 - Complete neglect of self-care with associated impairment in physical status
 - Extreme disruption in vegetative function (eg, life-sustaining functions such as eating)
 - Nearly complete inability to maintain any appropriate school behavior or academic achievement
 - Severely diminished ability to assess consequences of own actions (eg, acts of severe property damage)
 - Other evidence of severe dysfunction

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7. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.

Sufficient relief

- Sufficient relief is that which is adequate to alleviate the immediate impetus for recurrence of suicide attempt or act of harm to self or another.(1)(2)(3)

References

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Suicide attempt

- A suicide attempt may be identified by an actual, interrupted, or aborted act of self-injurious behavior accompanied by **1 or more** of the following(1)(2)(3):
 - Explicit or implicit evidence that the person believed the act would be lethal (eg, gaining access to gun, carbon monoxide inhalation)
 - Explicit or implicit evidence that the person intended to die (eg, act that was unlikely to be discovered in time for rescue, writing suicide note)

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Thoughts of suicide

- Thoughts of suicide or suicidal ideations are defined as "thoughts of serving as the agent of one's own death," to be distinguished from thoughts of death that do not involve actively bringing death about. (1)(2)

References

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